MINISTRY OF HEALTH & WELLNESS

CONTACT INVESTIGATION

FIELD GUIDE



2025

THE CONTACT INVESTIGATION FIELD GUIDE

Ministry of Health & Wellness, Jamaica

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ACRONYMS

ATL: Adult T Cell Leukemia
Bd: "Bis in die" (twice daily)
BPG: Benzathine Penicillin G
CC: Case Closure
CF: Case File
CI: Contact Investigator
CMV: Cytomegalovirus
CP: Critical Period
DIA: Direct Interview Approach
DRE: Digital Rectal Examination
EMTCT: Elimination of Mother to Child Transmission
EPI: Epidemiology
FR: Field Record
FTA-ABS: Fluorescent Treponemal Antibody Absorption Test
GUD: Genital Ulcer Disease
HAM/TSP: Human T-Lymphotropic Virus Type 1-Associated
HBsAg: Hepatitis B Surface Antigen
HIV: Human Immunodeficiency Virus
HIVST: HIV Self-Test
HPF: High Power Field
HTLV-1: Human T-Lymphotropic Virus Type 1
IC: Index Case
IM: Intramuscular
IPV: Intimate Partner Violence
IR: Interview Record
Kg: Kilograms
KP: Key Population
MHA-TP: Microhemagglutination Assay for *Treponema pallidum*
MO(H): Medical Officer (Health)
MOPD: Medical Outpatient Department
MSSR: Monthly Summary Statistical Report
MU: Million Units
NSU: National Surveillance Unit
OBGYN: Obstetrician and Gynaecologist
ON: Ophthalmia Neonatorum
PCI: Parish Contact Investigator
PID: Pelvic Inflammatory Disease
PLHIV: Person Living with HIV
PO: "Per os" (by mouth)
PrEP: Pre-exposure Prophylaxis

RCI: Regional Contact Investigator
S&S: Signs and Symptoms
SCI: Senior Contact Investigator
STAT: "Statim" (immediately)
STI: Sexually Transmitted Infections
STS: Serological Test for Syphilis
TPPA: Treponema pallidum Particle Agglutination Assay
TRUST: Toluidine Red Unheated Serum Test
VCT: Voluntary Counselling and Testing
VDRL: Venereal Disease Research Laboratory Test
WBC: White Blood Cell
WHO: World Health Organization

FOREWORD

The global health sector strategy to reduce the incidence of sexually transmitted infections (STIs) and other communicable diseases plays a crucial role in advancing the achievement of the 2030 Sustainable Development Goals. At the cornerstone of this initiative is the work of the contact investigator in the management of STIs in the Jamaica setting. Contact tracing, linkage to STI treatment, and risk-reduction counselling are three core responsibilities of the contact investigator, these are essential in mitigating the transmission of STIs and other infectious diseases in Jamaica**.**

To support these efforts, it is vital to have a standardized approach to contact investigation, ensuring that practices are aligned with the policies and procedures of the Ministry of Health and Wellness across the country. This guide is expected to promote standardization in the practice of contact investigation across the island, thereby enhancing outcomes and ultimately contributing to the reduction of sexually transmitted infections in Jamaica.

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PREFACE

Control of sexually transmitted infections (STIs), cannot be achieved by the treatment of cases presenting to care institutions, with signs or symptoms. Other public health measures are required to protect the susceptible population and break the chain of transmission.

Contact investigation plays a major role in the control of STIs, it is an acknowledged method for combating the spread of sexually transmitted infections. The strategy is to track sexual partners, while maintaining their confidentiality, and test and treat those who are infected. Each newly identified infected contact would then become the starting point (index case) of a new contact tracing process, continuing until no further contacts are identified.

Breaking the chain of transmission is crucial for controlling STIs. Transmission or reinfection is minimized through referral of sex partners for diagnosis and treatment. This activity may be undertaken by:

1. The Client – as a direct result of patient counselling and contact interviewing, which includes motivating the patient to assume responsibility for and play an active role in bringing contact(s) for evaluation and treatment;

   OR

2. The Health Provider (Assisted Partner Notification Services): "Refers to the process whereby HIV-positive clients, with their consent, receive support from a trained provider to disclose their HIV status or to anonymously inform their sexual and/or drug-injecting partner(s) of potential exposure to HIV. The provider then offers HIV testing to these partner(s). Assisted partner notification is done using contract referral, provider referral, or dual referral approaches."- WHO

CONFIDENTIALITY

Confidentiality is paramount in the work of a Contact Investigator, as it fosters trust and ensures individuals feel safe sharing sensitive information. By protecting privacy, CIs can effectively identify and notify contacts while maintaining the integrity of the process. The lack of confidentiality may contribute to social harm to the clients or their families, including instances of stigma, discrimination, and intimate partner violence.

Strategies to protect confidentiality include:

- Interviewing the patient or contact in private
- Coding data to protect the patient's diagnosis
- Storing the patient's data in a secure manner as per MOHW protocols for health records
- Ensuring access to patient records per protocols governing patient data

Confidentiality is a fundamental right. The Secrecy Act and the Data Protection Act govern all healthcare workers, including Contact Investigators, and these provide the framework for handling patient information. In the final analysis, the character, judgment, integrity, and sensitivity—essentially, the professionalism—of the Contact Investigator is the best guarantee of privacy and confidentiality. Alleged breaches in confidentiality are a real threat to the success of this aspect of the STI programme. Such instances should be investigated and appropriately managed to minimize harm to persons and the programme.

INTRODUCTION

This guide outlines the core activities involved in contact investigation—namely contact tracing, partner notification, testing, and provider referral. It is designed to standardize field operations while enhancing understanding of data collection tools, recording practices, and key terminologies.

It supersedes the *Contact Investigation Field Guide Manual (2020 edition)* and provides updated guidance on contact investigation, STI reporting, and associated laboratory procedures.

The 2025 revision includes several critical enhancements aimed at strengthening surveillance and programmatic reporting. These updates include:

- Revised STI Interview and Field Record Forms to improve data capture and standardization
- Updated Contact Investigation (CI) and STI Monthly Summary Statistics Report for enhanced monitoring
- Integration of HIV Self-Testing and Pre-Exposure Prophylaxis (PrEP) reporting
- Updated STI Testing and Clinical Management Algorithms aligned with current clinical guidelines
- Revised surveillance reporting forms to meet national reporting requirements

This guide also provides:

- Standardized forms for STI/HIV contact investigation and corresponding usage instructions
- Parish registers for structured recording of field data
- A list of priority infections and conditions for investigation
- Definitions and explanations of key terms
- Guidelines for submission of forms and reports
- Notes on related laboratory procedures
- Routing protocols for report submission
- A visual organogram of the Contact Investigation structure

FORMS, RECORDS, AND REPORTS USED TO GUIDE THE INTERVIEWING AND INVESTIGATION PROCESS

Background

The forms and reports used by CIs daily are essential to evaluate and guide programmatic decisions at the parish, regional, and national levels. The STI Interview Record and STI Field Record are described in terms of their purpose, routing, and how the forms should be completed.

STI Interview Record (IR) Form

Purpose

The Interview Record provides the information used for summary statistics. It is used to:

a) Compile information about the patient's contacts and suspected contacts for analytical and planning purposes.

b) Inform the parish surveillance team about the characteristics of patients diagnosed with priority infections.

c) Help CIs prepare the Monthly Summary Statistical Report (MSSR).

d) Provide a permanent record of interviews performed and their outcome for future investigations as necessary.

The interview record is used for interviewing the index case.

Routing

The Interview Record (IR) is completed in triplicate. The first copy is designated for the Index Case (IC), the second for Case Closure (CC), and the third for the Case File (CF).

1. The IC is submitted to the parish CI supervisor following the original interview.

2. The CC is submitted when the CI and the supervisor agree that the case is closed.

Note: Cases are generally closed when all of the Field Records (FRs) for the cases have been dispositioned.

The supervisor will set a cut-off time for closing the IR after making at least four reasonable attempts to conduct an investigation. This typically includes a minimum of three calls and at least three visits to the location, usually within a three-month period.

3. The CF copy is a permanent record to be retained by the CI.

How should the IR be completed?

The IR is completed using information obtained from the Direct Interview Approach (DIA) Worksheet and the Medical Record (docket). If more contacts or suspects are named than can fit in the space provided on the IR, a similar copy should be attached with the words "second copy" written across the top of the page. If re-interviews are performed, the additional information can be entered on the original CC form; the new information should be clearly noted and dated. Eventually, all sites will be equipped with tablets to digitize the interview records. This will give real time access to data and reports.

CI Form Digitization

With the digitization of the interview record, the combined forms for the index case (IC), case closure (CC), and case file (CF) are to be completed by the contact investigator, with provisions for the parish CI supervisor to pull reports for review and monitoring of the investigation of each case.

The following is a sample of the Interview Record form, accompanied by a detailed field-by-field explanation.

Interview Record Form

Contact Investigator Number/Name: _____ Parish: _____

STI Case Interview Record

Interview date: ___/____/_____ STI Code: _____

(dd/mm/yyyy)

Medical record #: _____ Case #: _____

(Number assigned to the case by the CI)

Last name: _____ First name: _____ MI: ____ Pet name/Alias: _____

Sex at Birth: _____ DOB: ___/___/_____ Age: _____

(dd/mm/yyyy) (Circle age if estimated)

Telephone #: _____ Address: _____

_____

Time at address_____ Time in area: _____ Travel: _____

Living with: _____ Other locating information/work: _____

Locations frequented: _____

Recreational activities: _____

Next of Kin/Emergency contact information

Last name: _____ First name: _____ Pet name/Alias: _____

Address: _____ Telephone #: _____

_____

Reason for interview: Antenatal ☐ Reactor ☐

Key Population: yes ☐ No ☐

If yes, specify_____

Where was the case interviewed? Clinic ☐ Hospital ☐ Field ☐

Symptoms Onset Date:	Duration (Days):	Description:
___/___/_____ (dd/mm/yyyy)	_____	_____
___/___/_____ (dd/mm/yyyy)	_____	_____

Contact Period Start Date: ____/____/_____ (dd/mm/yyyy)

Tested for Syphilis: Yes ☐ No ☐ Refused ☐

If yes, indicate: SD Bioline Result: _____ Date: ____/____/_____
(dd/mm/yyyy)

TRUST/VDRL Result: _____ Date: ____/____/_____
(dd/mm/yyyy)

Other (specify) Result: _____ Date: ____/____/_____
(dd/mm/yyyy)

Tested for HIV: Yes ☐ No ☐ Refused ☐

If yes, indicate the type of HIV test used and if the outcome was positive:

Type of test	Positive Results	
	Yes	No
HIVST ☐		
HIV Determine ☐		
HIV Unigold ☐		
ELISA ☐		

Indicate the final HIV test result: Positive ☐ Negative ☐ Indeterminate ☐ Pending ☐

Date of final HIV test result: ____/____/_____

(dd/mm/yyyy)

Rx: _____ Date: ____/____/_____

(dd/mm/yyyy)

_____ Date: ____/____/_____

(dd/mm/yyyy)

_____ Date: ____/____/_____

(dd/mm/yyyy)

Number of sex contacts named: _____

Critical Period Contacts Names and Addresses	Date LSI	Sex	FR Initiated	Parish Worked

Cluster Suspect's Names and Addresses	Sex	FR Initiated	Parish Worked

Sexual Health Education (indicate the counselling topics covered)

☐ Severity and potential complications of the disease

☐ Modes of transmission through sexual contact

☐ Recognizing signs and symptoms of the infection

☐ Lack of natural immunity against the disease

☐ Increased risk of acquiring other STIs

☐ Importance of consistent and correct condom use

☐ Recommendation for all sexual partners to be tested and treated

Pre-exposure prophylaxis:

Is the index case currently taking PrEP? Yes☐ No☐

Was the index case referred for PrEP? Yes☐ No☐

Was the case dispositioned? Yes☐ No☐

If yes, indicate the Date Closed: ____/____/_____

(dd/mm/yyyy)

If no, state the reason: _____

Notes:

_____
_____
_____
_____
_____

STI Interview Record - A Field by Field Explanation

CI #/NAME

- Enter the four-character CI code e.g.: SE02 then indicate CI name

PARISH

- Enter the three letter parish code for the parish where the patient lives e.g.: St Thomas = STT

INTERVIEW DATE

- Enter the date (DD/MM/YY) when the interview occurred

STI CODE

- Enter the STI code.

All of the patient's infections must be written on a single Interview Record Form

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Note: A full list of the STI codes is available on page 54.

MEDICAL RECORD NUMBER

- Enter the patient's medical record (docket) number

CASE #

- Enter the three letter parish code plus the next number in the sequence of cases handled by the CI

LAST NAME

- Enter the patient's last name

FIRST NAME

- Enter the patient's first name

MI

- Enter the patient's middle initial

PET NAME/ALIAS

- Enter any aliases used by this patient or names that other people call him/her

SEX AT BIRTH

- Enter the patient's sex assigned at birth ("**M**" = Male, "**F**" = Female)

DOB

- Enter the patient's date of birth (DD/MM/YY)

AGE

- Enter the patient's age. *If estimated, please circle the age*

TELEPHONE NUMBER

- Enter the patient's telephone number.

ADDRESS

- Enter full address for this patient or "UNKNOWN" if the address is unavailable

TIME AT ADDRESS

- Enter the time living at address

TIME IN AREA

- Enter the time living in location/community

TRAVEL

- Places where the person has visited and had a sexual encounter in Jamaica or another country during the critical period

LIVING WITH

- Person with whom they cohabitate, specify the relationship

OTHER LOCATING INFORMATION/WORK

- Where else the person can be found

LOCATIONS FREQUENTED

- Enter all areas that the patient frequently visits

RECREATIONAL ACTIVITIES

- Enter all the patient's hobbies and leisure activities inclusive of drug use

Explanations must be inserted for 'Time at Address', 'Time in Area', 'Travel', 'Living with', 'and Other Locating/Work', 'Locations Frequented' and 'Recreational Activities'

NEXT OF KIN/EMERGENCY CONTACT INFORMATION

- Enter information for the closest living relative to be contacted in case of emergency or for official matters related to the patient.

REASON FOR INTERVIEW

- Place a mark (X) in only one box to indicate the reason the patient is being investigated.

KEY POPULATION

Place a mark (X) in only one box [YES] or [NO] to indicate if the patient identifies as a member of the following key population groups:

1. Men who have sex with men (MSM)
2. Transgender people
3. Sex workers (including female, male, and transgender sex workers)
4. People who use drugs
5. People in prisons and other closed settings

WHERE WAS THE CASE INTERVIEWED

- Place a mark (X) in only one box to indicate whether the interview occurred in the [FIELD], at the [HOSPITAL], or [CLINIC]

SYMPTOMS

- List the ONSET DATE, DURATION (DAYS), and DESCRIPTION of all symptoms (e.g., rashes, sores, etc.) described by the patient for the illness for which he/she is being interviewed.

CONTACT PERIOD START DATE

- Enter the date when the patient's interview process commenced. The interview period is determined by selecting a significant date that is prior to the start of the critical period. The critical period refers to the maximum duration during which a patient may have been infected and, consequently, may have transmitted the infection to others. The critical period is the incubation period plus the duration of signs and symptoms described

TESTED FOR SYPHILIS

- Place a mark (X) if the patient was tested for SYPHILIS [YES], or was not tested for SYPHILIS [NO], or if they refused to be tested. [REFUSED]. If a test was done, the result must be entered when the IR is closed.

- If the patient was tested for SYPHILIS, indicate the type of test used (SD Bioline only or SD Bioline and TRUST/VDRL), the result(s) and the date of the test. They should be entered on the case closure and case file copies of the IR at the time of closure.

TESTED FOR HIV

- Place a mark (X) if the patient was tested for HIV [YES], or was not tested for HIV [NO] or if they refused to be tested. [REFUSED]. If a test was done, the result must be entered when the IR is closed.

- If the patient was tested for HIV, place a mark (X) to indicate the type of test used (HIV Self-test, HIV Determine, HIV Unigold, or ELISA). If the test used was positive place a mark (X) in the field [YES] or [NO].

- Place a mark (X) to indicate the patient's final HIV test result (positive, negative, indeterminate, or pending) and write the date of the result. This result must be entered when the IR is closed.

RX (TREATMENT INFORMATION)

- Enter the treatment (name and dosage of drug) and the date the treatment was started for the infection listed on the interview record

NUMBER OF SEXUAL CONTACTS NAMED

- This is the number of contacts that the patient had sexual contact with during the period before the interview. It is NOT the number initiated on the Field Record. *This is an important distinction*.

CRITICAL PERIOD CONTACTS' NAMES AND ADDRESSES

- Enter names of sex partners (contacts) during the critical period that are provided by the interviewed patient. Enter the date of last sexual intercourse in the "DATE LSI" column for each contact named. Place an "M" or "F" in the "SEX" column. If a field record (FR) was initiated, place a "Y", if not place "N" in this column.

- **N.B.** Unless otherwise approved by your supervisor, it is assumed that ALL critical period contacts will be initiated unto Field Records. Include the "PARISH TO BE WORKED" for each name provided, using the three-digit parish code.

CLUSTER SUSPECTS' NAMES AND ADDRESSES

- Enter names of cluster suspects (non-sex partners outside of the critical period) that are provided by the interviewed patient.

- Place an “M” or “F” in the “SEX” column. If a field record (FR) was initiated, place a “Y” in this column. Include the “PARISH TO BE WORKED” for each name provided, using the three-digit parish code.

PARISH TO BE WORKED

- Whenever a critical period contact(s) is named whose address(es) is not in the same parish where the interview is taking place, the parish to which the FR will subsequently be sent should be indicated (This is the parish where the contact will be investigated).

SEXUAL HEALTH EDUCATION

- These are reminders at the bottom of the IR form (e.g., serious, sexually transmitted, etc.) used for education and counselling. Each reminder highlights an STI topic that the patient should be informed about. Place a tick in the box for each item covered during the interview.

PRE-EXPOSURE PROPHLYAXIS (PrEP)

- Place a mark (X) if the patient was taking PrEP for HIV at the time of disease acquisition [YES], or was taking PrEP at the time of disease acquisition [NO].

- Place a mark (X) if you referred the patient for PrEP [YES], or did not refer the patient for PrEP [NO].

DISPOSITION

- Indicate with a (X) if the case was dispositioned/closed [YES], or not dispositioned [NO].

- If the case was dispositioned, state the date the case it was closed.

- When all contacts and suspects have been examined/ dispositioned, the case may be closed with supervisory concurrence. Attach any closed Field Records not yet submitted to the supervisor at the back of the second copy of the IR and submit this to the parish supervisor. The supervisor will review and append his initials to the date if he agrees that the case is completed. Regional/Senior Parish CIs should audit these forms on a quarterly basis.

- If the case was not dispositioned, state the reason.

N.B. An IR should always be submitted when a patient is diagnosed with a priority infection in the CI work area, regardless of whether the patient was interviewed or not. Special report forms are

required for pediatric STIs.

NOTES

- Provide any additional notes relevant to the patient's interview on the reverse side of the form, as needed. Include any follow-up plans, if applicable.

STI Field Record (FR)

Purpose

The STI Field Record:

a. Is a guide to the investigation in the field

b. Informs the CI and their supervisor about work to be done (open work) and work completed (closed work)

c. Provides a permanent record for the CI for future reference

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The STI Field Record Form is for the investigation of contacts

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How does the FR guide investigations?

The FR is initiated when contacts and suspects are identified during interviews with priority patients or when laboratory reports or rumours suggest a need to notify suspected cases. It contains all the information the CI requires to locate the individual, confirm their identity, and advise them about necessary medical follow-up.

In the case of laboratory test follow-up, the information for the FR is transcribed from laboratory reports. For contacts and suspects, the information is typically gathered by the CI from an infected patient. In some instances, other healthcare providers may identify contacts and refer them to a CI for follow-up. Consistent with the practice that FRs open investigations, these contacts should also be recorded on FRs and handled in the same manner. The key difference is that there is no interview record to associate with the FR in these cases.

The following is a sample of the STI Field Record form, accompanied by a detailed field-by-field explanation.

STI Field Record Form

Contact Investigator Number/Name: _____ Parish: _____

STI Field Record (Contact)

Date of Initiation: ____/____/_____ Original Patient Care #: _____

(dd/mm/yyyy) (Refer to Case Interview Record)

Date Assigned: ____/_____/_____ STI code(s): _____

(dd/mm/yyyy)

Last name: _____ First name: _____ MI: ____ Pet name/Alias: _____

Sex at Birth: _____ DOB: ___/___/_____ Age: _____

(dd/mm/yyyy) (Circle age if estimated)

Is the client pregnant: Yes ☐ No ☐ Not Applicable ☐

Address: _____ Telephone #: _____

_____

Other locating information & description:

_____

_____

Reason for contact investigation: Contact ☐ Suspect ☐ Cluster☐

Surveillance ☐ Laboratory referral ☐

Exposure frequency ☐ Exposure dates: _____/_____/_____ to _____/_____/_____

(dd/mm/yyyy) (dd/mm/yyyy)

Is the client a member of a key population group? Yes ☐ No☐

If yes, specify _____

Contact investigation start date: ____/____/_____

(dd/mm/yyyy)

Where was the client seen: Clinic ☐ Hospital ☐ Field ☐

If the client was seen in the field, specify the number of field visits

0 ☐ 1☐ 2☐ 3 ☐ 4☐

Syphilis Test:

Tested for Syphilis: Yes ☐ No ☐ Refused ☐

If yes, indicate: SD Bioline Result: _____ Date: ____/____/_____

(dd/mm/yyyy)

TRUST/VDRL Result: _____ Date: ____/____/_____

(dd/mm/yyyy)

Other (specify) Result: _____ Date: ____/____/_____

(dd/mm/yyyy)

Newly diagnosed ☐ Previously diagnosed ☐ Not infected ☐

Diagnosis: A1 ☐ A2 ☐ A3 ☐ A6 ☐

HIV Test:

Tested for HIV: Yes ☐ No ☐ Refused ☐

If yes, indicate the type of HIV test used and if the outcome is positive:

Type of test	Positive Results	
	Yes	No
HIVST ☐		
HIV Determine ☐		
HIV Unigold ☐		
ELISA ☐		

Indicate the HIV final test result: Positive☐ Negative ☐ Indeterminate ☐ Pending ☐

Date of final HIV test result: ____/____/_____

(dd/mm/yyyy)

Newly diagnosed ☐ Previously diagnosed ☐ Not infected ☐

Case outcome: ☐ Worked ☐ Not worked

If worked, state the progress:

☐ Unable to locate

☐ Located, refused test

☐ Located, did not report to the facility for testing

☐ Located, refused treatment

☐ Located, tested but not treated

☐ Located, tested and treated

If not worked, state the reason below

_____

_____

Disposition date: ____/____/_____

(dd/mm/yyyy)

Date referred (out of parish): ____/____/_____

(dd/mm/yyyy)

Where was the case referred? _____

(specify the parish)

Which CI was the case referred to?_____

(specify the name)

Case outcome? _____

Notes:

_____
_____
_____
_____

STI Field Record - A Field by Field Explanation

CI#/NAME

- Enter the four-character CI code or name for the worker assigned this FR

DATE OF INITIATION

- The purpose of this entry is to determine when the area received the work, which is not necessarily the date that the CI received the work. Enter the date (DD/MM/YY) the supervisor received this FR. If the supervisor did not initially receive the record, provide the date the field CI received it.

ORIGINAL PATIENT CASE

- The original patient (index case) number. The purpose of this entry is to relate the FR to the Interview Record. Enter the three-letter parish code followed by the unique case number listed on the related Interview Record. This field must be completed if the STATUS is either CONTACT or SUSPECT. This field must be left blank if the STATUS is POSSIBLE CASE (for example from a laboratory referral). The reason for the latter is that the information did not come from an interview of an infected patient and therefore cannot be related to an IR.

DATE ASSIGNED

- The purpose of this entry is to determine the first date when the CI had an opportunity to begin work. This information can be used to calculate how long it takes the CI to complete the work by comparing this date with the closure date. Enter the date (DD/MM/YY) when the CI assigned to this FR receives the FR to be worked.

STI CODE

(See page 54)

LAST NAME

- Enter the patient's last name

FIRST NAME

- Enter the patient's first name

MI

- Enter the patient's middle initial

PET NAME/ALIAS

- Enter any aliases used by this patient or names that other people call him/her

SEX AT BIRTH

- Enter the patient's sex at birth ("**M**" = Male, "**F**" = Female)

DOB

- Enter the patient's date of birth (DD/MM/YY)

AGE

- Enter the patient's age. If estimated, please circle the age

ADDRESS

- Enter full address for this patient or "UNKNOWN" if the address is unavailable

OTHER LOCATING INFORMATION/DESCRIPTION

- Enter any additional locating or physical description or information available for this contact/person. If there is not enough room in the space provided, write the extra information on the back. Enter any source of information (laboratory data, rumours/hearsay etc.) for POSSIBLE CASE here.

- POSSIBLE CASE – A person with a reported reactive serology, usually from a laboratory, antenatal centre, or blood bank.

TELEPHONE #

- Enter the contact's/person's telephone number (if available)

REASON FOR CONTACT INVESTIGATION

- Place a mark (X) in only one box to indicate whether the subject of this FR represents a [CONTACT], [SUSPECT], [CLUSTER], [SURVEILLANCE] or a [LABORATORY REFERRAL]. If this represents a possible case, then provide the laboratory results that are known for this patient.

- SUSPECT AND CLUSTER SUSPECT- Any sex contact named by the original interviewee (index case) but who is outside the critical period is a "suspect". A "cluster suspect" is a named non-sex partner who:

    - Has lesions and/or rashes that are suggestive of syphilis.

    - May be a sex partner of a known or suspected case of syphilis.

    - May benefit from an examination for syphilis.

EXPOSURE FREQUENCY

- Enter the frequency of sexual contact, if known, as described by the infected partner e.g., 1x/week, 5x/month etc.

EXPOSURE DATES

- Enter the first and last dates (DD/MM/YY) of exposure for all exposed contacts. Only enter dates when the REASON FOR THE CONTACT INVESTIGATION is a CONTACT.

- CLUSTER SUSPECTS who had contact with the infected patient outside of the critical period will have no date entered here. This is noted under OTHER LOCATING INFORMATION & DESCRIPTION.

KEY POPULATION

- (see page 20)

CONTACT INVESTIGATION START DATE

- Enter the date (DD/MM/YY) that the contact investigation started for the reason that this FR was initiated. If test and treatment were given on the same date, it may also be the disposition date. The examination date is only completed when the person is located and examined. The

reason for this, is that this date will be used to calculate the time interval between the time the CI received the work and the first date that the person was examined.

NO. OF FIELD VISITS

- Tick the appropriate box indicating the number of visits made in order to locate and disposition the contact/person. Tick "0" if no visits made. Examples of no visits are when a contact is already in the clinic, when the patient is diagnosed, or when the patient has notified the contact (patient referral).

OTHER LOCATING INFORMATION/DESCRIPTION

- Enter any additional locating or physical description or information available for this contact/person. If there is not enough room in the space provided, write the extra information on the back. Enter any source of information (laboratory data, rumours/hearsay etc.) for POSSIBLE CASE here.

WHERE WAS THE CLIENT INTERVIEWED

- Place a mark (X) in only one box to indicate whether the interview occurred in the [FIELD], at the [HOSPITAL] or [CLINIC]

- If the [FIELD] was selected, indicate the number of field visits made.

SYPHILIS TEST

- Place a mark (X) if the patient was tested for SYPHILIS [YES], or was not tested for SYPHILIS [NO], or if they refused to be tested. [REFUSED].

- If the patient was tested for SYPHILIS, indicate the type of test used (SD Bioline only or SD Bioline and TRUST), the result(s), and the date of the test.

- Indicate if the client is newly diagnosed, previously diagnosed with syphilis or not infected with syphilis. A syphilis diagnosis is made using the client's medical history, signs and symptoms, and laboratory results.

- Place a mark (X) in only one box to indicate the client's stage of syphilis (see page 54 for the STI coding).

HIV TEST

- Place a mark (X) if the patient was tested for HIV [YES], or was not tested for HIV [NO] or if they refused to be tested. [REFUSED].

- If the patient was tested for HIV, place a mark (X) to indicate the type of test used (HIV Self-test, HIV Determine, HIV Unigold, or ELISA). If the test used was positive place a mark (X) in the field [YES] or [NO].

- Place a mark (X) to indicate the patient's final HIV test result (positive, negative, indeterminate or pending) and write the date of the result. If an HIV test was done, the final result must be entered when the FR is closed.

- If the client is positive for HIV, indicate if this is a newly diagnosed or previously diagnosed case. If the client tested negative for HIV, indicate not infected.

CASE OUTCOME

- Indicate with a mark (X) if the case was worked or not worked. For cases not worked, indicate the progress of the case by placing a mark (X) by either of the following options [UNABLE TO LOCATE], [LOCATED, REFUSED TO TEST], [LOCATED, DID NOT REPORT TO THE FACILITY FOR TESTING], [LOCATED, REFUSED TREATMENT], [LOCATED, TESTED BUT NOT TREATED or [LOCATED, TESTED AND TREATED]

- **If the case was [NOT WORKED] an explanation or reason must be given.**

DISPOSITION DATE

- Enter the date (DD/MM/YY) that a disposition was determined for this FR. For syphilis seropositive clients, the disposition date is the when client has completed treatment. For HIV-positive clients, the disposition date is the date the client was tested for HIV.

DATE REFERRED (OUT OF PARISH)

- Enter the date the FR was referred out of the parish and specify which parish it was referred to, so that it may be handled by another CI. In such cases, do not make any entries after the [CONTACT INVESTIGATION START DATE]. Once the case has been worked, these field records must be sent to the supervisory CI, accompanied by a note indicating the parish(es) from which they were referred, to ensure proper linkage to the index case.

- Ensure that the case is followed-up and state the outcome of the contact investigation process.

The STI Field Record Reporting Mechanism or Routing

There are three copies of the Field Record. All three copies (first, second and third) of the FR are initiated at the same time by inserting carbon paper between the copies (or copies may be carbonized).

a. The *first copy* of the FR:

- For contacts and suspects - Attach the first copy of the FR to the first copy of the Interview Record and submit them together through the parish supervisors to initiate the investigation.

- For 'possible cases' – Submit the first copy of the FR separately as there is no interview record. Check with the supervisor to see if the copies of 'possible cases' should be catalogued with the weekly submissions of the closed second copies.

b. The *second copy* of the FR:

- Dispositioned (closed) copies of second FRs are submitted to the supervisor weekly.

- At times, the first and second copies can be dispositioned simultaneously.

- For example, a patient with early syphilis that voluntarily brings their contact to the clinic. In that case, the second FR for the partner can also be attached to the IR, but the 'NO. OF VISITS' field should be marked as '0'.

- The CI should update the information on closed copies of FRs, ensuring that the original information remains intact. This updated information is used by the parish supervisor to identify which first copy is being closed.

- At the time a case is closed, all FRs pertaining to that case and not already submitted, must be attached to the closed copy of the IR and submitted.

c. The *third copy* of the FR:

- The third copy of the FR is crucial for infection control, as it contains information for follow-up. For this reason, special care should be taken to ensure it is legible before releasing the copies, to avoid the risk of losing important information.

- The third copy is used in the field and should be securely filed, such as in a small ring binder. Notes on the actions taken can be recorded on the back of the third copy. Once dispositioned, the third copy should be retained in a permanent file for reference in future investigations.

Summarized Procedures for Completion and Submission of Interview and Field Record Forms

1. Interview record forms are provided in triplicate. The first copy is referred to as the Index Case (IC); the second copy as Case Closure (CC) record and the final copy as the Case File (CF) record. On completion of the interview record, all contacts and suspects named with locating information will be transcribed to Field Record forms for contact tracing. The IC copy of the IR will then be submitted to the Parish CI for review. On receipt, the Parish CI will review all forms for final diagnosis.

2. A FR form will be completed for each contact named with locating information. This will initiate the investigation on that contact. Periodically (weekly), all dispositioned FRs will be submitted to the Parish CI. When a case is closed, all FRs pertaining to that case (and any cases not already submitted to the Parish CI) must now be attached to the CC copy of the IR and submitted.

3. All FRs dispositioned as "NOT WORKED" must have a reason as required on the form. Contacts referred from other parishes must be investigated, dispositioned and forwarded to the Parish CI as soon as possible. These Field Records must indicate which parishes the contacts were referred from so that they can be linked to the index case.

4. A Field Record must be initiated whenever a syphilis serology result indicates a possible case. E.g., from ANC screening or the Blood Bank. The status for this record is POSSIBLE CASE. When the FR is created, the first copy is submitted to the Parish CI as above. When the FR is dispositioned, the second copy is also forwarded to the Parish CI.

5. When a case is closed (i.e. when all CONTACTS and SUSPECTS named have been dispositioned), the form must be filed by the Contact Investigator. The names of contacts given by patients who are to be located outside of the parish (e.g. contacts to persons diagnosed with A1 or A2 residing in other parishes) must be forwarded to the appropriate Parish CI for contact tracing and the necessary reporting.

* Changes to procedures are pending the digitalization of Interview and Field records.

Reports and Reporting Flow

Figure 1: Flow of STI and CI Monthly Summary Statistic Report (MSSR)

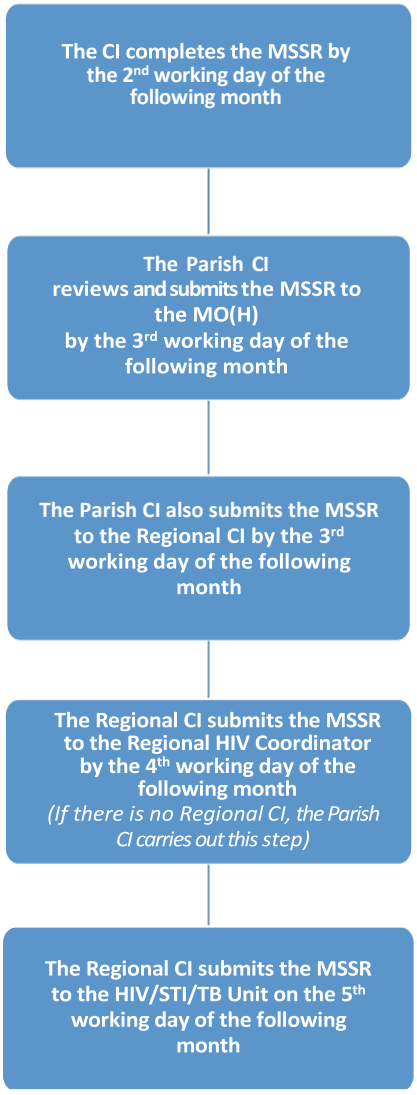
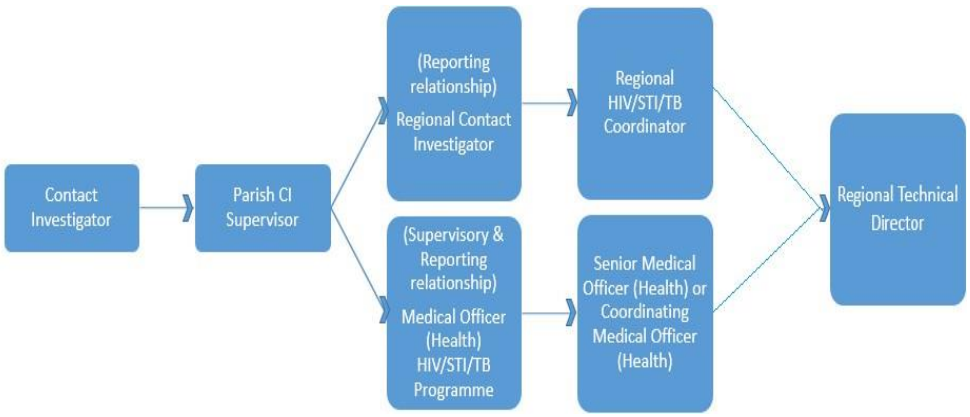


Figure 2: Contact Investigators' Organogram



1. Each Contact Investigator must compile a Monthly Summary Statistics Report, including general comments on the prescribed form, and submit it to the Parish CI Supervisor. The report reflects any additional significant observations or necessary information that is important to the programme but is otherwise not captured on the IR or FR forms.
2. The Regional Contact Investigator (RCI) is required to submit a monthly report to the respective regional focal point (HIV Coordinator) and central focal point (Programme Development Officer, MOHW) using the prescribed CI reporting format (pages 41–45) and STI reporting format (page 56-58). This report is due on the 5th working day of the following month. It must be submitted on time; if there is a delay, an explanation must be provided via email. Annual and biannual performance evaluation reports must be submitted for each Contact Investigator (CI), as required.
3. The Parish CI is required to receive and check that the following reports are properly completed:
    a. All IRs and FRs.
    b. CI and STI MSSRs before having them entered in the Central programme.
    c. Special Paediatric Reports
    d. Produce monthly and/or quarterly summaries of the above reports as required
4. The Parish CI must also submit a monthly summary report of activities (field visits, staff meetings attended, concerns and actions taken etc.) to the Medical Officer (Health), in time for the monthly STI Team meeting and using an appropriate format. Annual and half-yearly performance evaluation reports (as required) must be submitted to each regional CI.
5. All investigators must remember their obligation to submit reports to the Medical Officers

(Health) and to attend parish, regional, and central meetings on a timely and regular basis.

The Contact Investigator and the STI Database

The monthly CI and STI reports are entered into the District Health Information 2 (DHIS2) database, which includes both a CI report and an STI report. The data should be reviewed by the Senior CI and the Medical Officer of Health before being submitted to the national level at the HIV/STI/TB Unit within the Ministry of Health and Wellness.

The Contact Investigation MSSR consists of eight (8) sections, these are:

1. General Indicators
2. HIV Testing Disaggregation
3. HIV/AIDS Death Disaggregation
4. New Cases of Priority Conditions
5. Paediatric indicators
6. Outreach indicators
7. Entry to Care Indicators
8. PrEP indicators

The STI MSSR consists of four sections, these are:

1. Syphilis
2. STI Syndromes
3. Other STI and Non-STI Conditions
4. STI Clinic Data

Completion of Forms

Individuals responsible for data entry of record forms are not always Contact Investigators. Therefore, it is essential that the Contact Investigator ensures all fields on the record are accurately completed. The data entry clerk can only input the information that has been recorded. For example, if the field for the date is left incomplete and the supervisor fails to make the necessary correction, the record will either be sent back to the Contact Investigator for revision, or the field will remain empty. Returning record forms is time lost. If the field is left blank, then calculating the time to trace the contact is not possible.

Some Key Definitions

CRITICAL PERIOD (CP)

Estimating the Critical Period (CP) is critical for identifying "high-priority" contacts. It is also used to determine which contacts of patients diagnosed with primary or secondary syphilis are eligible for and require epidemiologic (EPI) treatment.

1. For Primary Syphilis- Calculate the critical period by adding the maximum incubation period

(IP) of 90 days to the duration of signs and symptoms (S&S) present before treatment.

2. For Secondary Syphilis – Calculate the critical period by adding the maximum incubation period of six months to the duration of signs and symptoms present before treatment.

CRITICAL PERIOD CONTACT

1. A contact exposed to infectious (usually primary and secondary) syphilis during the critical period for the stage of the disease. (CP = maximum IP + duration of S&S). Maximum IP for common STIs are tabulated below. To each, add the duration of signs and symptoms prior to treatment to determine the period of infectivity, i.e. critical period.

Table A: Maximum Infectious Period for Common STIs

STI	**Maximum Infectious Period (months)**
Primary syphilis	3
Secondary syphilis	6
Early latent syphilis	24
HIV	Indefinitely
Gonorrhea	1
Chlamydia	3

The Contact Investigation Programme

In addition to the STI MSSR, the CIs are responsible for reporting on contact investigation processes through the Contact Investigation Monthly Summary Statistics Report (CI MSSR). Contact Investigators are also required to submit several supplementary reports related to contact investigations of priority diseases.

The following page is a sample of the CI MSSR form, accompanied by an explanation of key terms.

CONTACT INVESTIGATION PROGRAMME MONTHLY SUMMARY STATISTICS REPORT

CI #: **Month:_____** **Parish: _____**

CI Name:_____ **Year:_____** **Region**: _____

General Indicators	Syphilis						HIV			AIDS			HIV/AIDS Surveillance			AIDS Death		
	Primary & Secondary			Early Latent														
	M	F	Total	M	F	Total	M	F	Total	M	F	Total	M	F	Total	M	F	Total
Number of new cases reported			0			0			0			0			0			0
Number of new cases interviewed			0			0			0			0			0			
Number of contacts named			0			0			0			0			0			0
Number of contacts locatable			0			0			0			0			0			0
Number of contacts located			0			0			0			0			0			0
Number of contacts tested			0			0			0			0			0			0
Number of contacts positive			0			0			0			0			0			0
Number of contacts known positive			0			0			0			0			0			0
Number of positive contacts who are newly diagnosed			0			0			0			0			0			0

HIV Index Testing Disaggregation	M	F	Total
Number of children (0-17 years) listed by index clients			0
Number of index clients' children (0-17 years) tested for HIV			0
Number of index clients' children (0-17 years) positive for HIV			0
Number of index clients' children (0-17 years) known HIV-positive			0
Number of index clients' children (0-17 years) newly diagnosed with HIV			0
Number of HIV-negative partners linked to prevention (condoms & PrEP)			0
Number of index clients screened positive for IPV			0

HIV/AIDS Deaths Disaggregation	M	F	Total
AIDS Related Deaths			0
Non-AIDS Related Deaths			0

New Cases of Priority Conditions Interviewed or Counselled by Contact Investigator			
	M	F	Total
Interviewed or Counselled			0
Genital Discharge Syndrome			0
Genital Ulcer Disease			0
Pelvic Inflammatory Disease			0

Paediatric Indicators	Congenital Syphilis (CS) Exposed Infants	HIV Exposed Infants (HEI)	Suspected cases of Ophthalmia Neonatorum (ON)
Number of new cases reported			
Number of new cases investigated			
Number of cases closed			

Outreach Indicators	No. Done	Persons Reached
Talks/Group (2 or more persons) Counselling Sessions		
Individual counselling Sessions		
Targeted Outreach Sessions		
Field Visits		

Entry to Care CI Indicators	M	F	Total
Number of clients **Reached via Field Visits**			0
Number of clients who received 3 or more field visits and **Not reached**			
Number of clients **Reached via Calls**			0
Number of clients who received 3 or more calls and **Not reached**			0
Total			0

PrEP	Population																							
	Multiple Sex Partners (MSP)								History of Sexual Transmitted Infections (HSTI)								Sero-Different (SD)							
	M				F				M				F				M				F			
Indicators	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+
Number of clients engaged for PrEP by the CI																								
Number of clients willing to initiate PrEP																								
Number of clients that commenced PrEP from those engaged by the CI																								
Number of clients newly initiated during the reporting period																								
Number of clients reinitiated on PrEP in the reporting period																								
Number of clients that continued PrEP in the reporting period																								
Number of clients on PrEP who tested positive for HIV during the reporting period																								
Number of clients who were diagnosed with a STI while on PrEP, during the reporting period																								

PrEP	Population																							
	Men who have Sex with Men (MSM)								Female Sex Workers (FSW)								Transgender (TG)							
	M				F				M				F				M				F			
Indicators	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+	16-24	25-34	35-49	50+
Number of clients engaged for PrEP																								
Number of clients willing to initiate PrEP																								
Number of clients newly initiated during the reporting period																								
Number of clients reinitiated PrEP in the reporting period																								
Number of clients continued on PrEP in the reporting period																								
Number of Clients who contracted HIV during the reporting period																								
Number of clients who were diagnosed with a STI while on PrEP, during the reporting period																								

The CI Monthly Summary Statistics Report – An explanation of key terms

General Indicators

Number of new cases reported

- Indicate the number of index cases occurring/referred to the parish for the first time.
- An index case is a person newly diagnosed with an infectious disease, such as syphilis, HIV, or AIDS, that is able to transmit the disease.

Number of new cases interviewed

- Indicate the number of index cases that were identified from contact tracing that were interviewed. This excludes persons that are mentally unstable and deceased.

Number of contacts named

- Indicate the total number of contacts elicited from cases interviewed (including contacts without proper names or locating addresses).
- The CI does not need to differentiate between sexes in recording contacts e.g. if. A male named six (6) contacts that figure should be placed under Male, and the same applies to the Female.

Number of contacts locatable

- Indicate the number of contacts with "locating" addresses/information.

Number of contacts located

- Indicate the number of contacts found of those locatable

Number of contacts tested

- Indicate the number of contacts tested from those that were located
- **N.B. Ratio Contact to Cases: (No. of contacts named divided by the No. of cases interviewed) and Percent of Contact Positive will be inserted by the computer programme**

HIV Disaggregation

Number of children (0-17 years) listed by index clients

- Indicate the number of the index clients' children aged 0-17 (this includes both alive and deceased children).

Number of index clients' children (0-17 years) tested for HIV

- Indicate the number of the index clients' children aged 0-17 years (this includes children who are alive and deceased) that were tested for HIV.

Number of index clients' children (0-17 years) positive for HIV

- Indicate the number of the index client's children aged 0–17 years (including both living and deceased) who were confirmed positive for HIV. This includes children newly diagnosed with HIV as well as those previously known to be HIV-positive.

Number of index clients' children (0-17 years) known to be HIV-positive

- Indicate the number of the index clients' children aged 0-17 years (this includes children who are alive and deceased) who were confirmed positive for HIV. This includes children who were previously known to be positive for HIV.

Number of index clients' children (0-17 years) newly diagnosed with HIV

- Indicate the number of the index clients' children aged 0-17 years (this includes children who are alive and deceased) that were confirmed positive for HIV. This includes children who are newly diagnosed with HIV.

Number of HIV-negative partners linked to prevention (condoms & PrEP)

- Indicate the number of contacts of index clients who tested HIV-negative and were provided with prevention methods such as condoms and PrEP.

Number of index clients screened positive for IPV

- Indicate the number of index clients screened positive for IPV using the Intimate Partner Violence risk assessment (see page 63.)

HIV/AIDS Deaths Disaggregation

Number of HIV/AIDS-related deaths

- Indicate the number of deaths that were due to HIV/AIDS-related causes, such as diseases caused by HIV/AIDS**.**

Number of non-HIV/AIDS-related deaths

- Indicate the number of deaths that were not due to HIV/AIDS related causes, such as trauma, gunshot wounds, etc.

New Cases of Priority Conditions Interviewed, Counselled or Tested by Contact Investigator

- Indicate the number of clients that were interviewed or counselled by the contact investigator for the following conditions:
    - Genital Discharge Syndrome
    - Genital Ulcer Disease
    - Pelvic Inflammatory Disease

Paediatric Indicators

Congenital Syphilis

- This includes the all infants born to syphilis seropositive mothers (SD-Bioline and TRUST reactive) who were not appropriately treated for syphilis.

---

*All syphilis-seropositive mothers must receive at least one (1) dose of **Benzathine Penicillin G** intramuscularly no less than 30 days before the delivery of their infant(s).*

HIV Exposed Infants

- This includes the all infants born to HIV positive mothers

Ophthalmia Neonatarum

- This includes all infants suspected to have ophthalmia neonatorum.
- All Class 1 notifiable diseases must be reported upon suspicion of the disease.

Outreach Indicators

Talks/Group Counselling Sessions

- Talks given to any group of 2 or more persons.

Individual Counselling Sessions

- One to one counselling e.g. each client seen in the STI clinic.

Targeted Outreach (Intervention) sessions

- Sessions with target groups e.g., CSW, MSM; Inner-city Youths, prisoners, etc.

Field visit

- Any visit done to perform official duties such as case-finding, contact tracing, or surveillance on the field.

Entry to Care CI Indicators

Number of clients reached via field visits

- Indicate the number of clients reached by field visits

Number of clients who received 3 or more field visits and not reached

- Indicate the number of clients who received 3 or more field visits and were not reached

Number of clients reached via calls

- Indicate the number of clients who were reached via calls

Number of clients who received 3 or more calls and not reached

- Indicate the number of clients who received 3 or more field visits and were not reached.

Pre-Exposure Prophylaxis (PrEP) Indicators

Number of clients engaged for PrEP

- Indicate the number of patients informed about PrEP, its benefits, where it can be accessed and its expected outcome.

Number of clients eligible for PrEP

- Indicate the number of persons that met the criteria for PrEP eligibility.
- Criteria for PrEP Eligibility:

    1. Be HIV Negative
    2. Be at substantial risk for HIV acquisition - target population and or demonstrate

substantial risk for acquiring HIV
3. Must not show signs/symptoms of an acute HIV infection
4. Must have a Creatinine Clearance of > 60 ml/min (for Oral Daily PrEP).

Number of clients linked to Sexual Reproductive Health services

- Indicate the number of clients on PrEP who were linked to sexual and reproductive health services, including the provision of family planning and condom distribution.

HIV Self Testing (HIVST)

HIV Self-Testing (HIVST) is offered through outreach initiatives and private service providers. OraQuick is the test kit used. Persons with a positive self-test result are encouraged to visit their nearest public or private care provider for counselling and testing using the national HIV testing algorithm.

Contact Investigator Programme Targets and Expectations

The following is a summary of the expectations and programmatic targets of the Contact Investigator:

1. Locate and interview individuals diagnosed with STIs, particularly syphilis and HIV/AIDS.
2. Support surveillance and screening protocols for STIs in clinical settings.
3. Identify and engage individuals eligible for Pre-Exposure Prophylaxis (PrEP).
4. Conduct contact tracing for all clients diagnosed with HIV, STIs, or TB.
5. Ensure early diagnosis, timely review, and linkage to care for all persons living with HIV (PLHIV) within four weeks of diagnosis.
6. Conduct annual follow-up with all PLHIV, eliciting new contacts for tracing as needed.
7. Re-interview all unsuppressed PLHIV clients every six months for updated contact tracing.
8. Interview PLHIV with drug-resistant HIV every six months to update contact lists.
9. Interview all patients returning to care and elicit contacts for tracing within five working days of their return.
10. Locate patients lost to follow-up from treatment sites within two weeks of referral.
11. Arrange testing for all HIV and STI contacts, and schedule them for retesting after the window period.
12. Provide risk-reduction counseling, distribute condoms, and ensure proper documentation.
13. Deliver post-test risk-reduction counseling for clients testing negative.
14. Ensure HIV and syphilis testing is provided to all individuals reporting an STI or requesting STI testing.
15. Assign all priority cases to a Contact Investigator (CI) and complete a field visit within 72 hours. All CIs must follow up on these priority cases, ensuring that treatment is completed and accurately documented. Priority STI cases include:

    1. Pregnant women with reactive serology
    2. HIV-positive individuals
    3. Infectious syphilis (including primary, secondary, and early latent stages)
    4. Neonatal (congenital) syphilis
    5. Ophthalmia neonatorum
    6. HIV/AIDS
    7. Sexually Transmitted Infections (STIs) in children

Performance and Reporting Standards:

Performance and reporting standards must be met for the management of all STIs, as seen below:

- The turnaround time for addressing priority STI cases should not exceed six weeks.
- All investigative outputs must be completed and submitted within six weeks.
    - At least 95% of new cases must be interviewed within six weeks.
    - A minimum of 90% of all locatable contacts must be located and tested within six weeks.

- All Class 1 notifiable illnesses must be reported within 24 hours of **suspicion** or confirmation.
- Confidential Reporting Forms and Surveillance Reporting Forms must be completed and submitted within 4 to 6 weeks. At a minimum, case information should be submitted within this timeframe.
- Field reports should be completed and submitted on the same day the client is interviewed.

Priority STIs/Conditions for Investigation

There are conditions classified as priority conditions and govern the approach to the investigation by the CI. These conditions/ diseases should receive priority for investigation. Examples include, A1, A2, A3 (especially in pregnant women), and C13 which must be given provider investigation priority. Other conditions should be given patient referral priority. Patients should be referred to a trained STI Contact Investigator for partner notification and management. These are reported on the STI Monthly Summary Statistics Report.

Priority A

1. HIV/AIDS (C13)
2. Pregnant women with reactive serology
3. Early congenital syphilis (< 1 yr.) (A7)
4. Infectious Syphilis (A1, A2, A3)
5. Ophthalmia Neonatorum
6. STI in Childhood

Priority B

1. Severe Pelvic Inflammatory Disease
2. Contacts/Referrals
3. Hepatitis B
4. Resistant Gonorrhea
5. Other Genital Ulcer Disease (GUD)

STI Coding

This coding helps to ensure the confidentiality of patient records and contact investigation.

1. New cases of Syphilis

   A1 Primary

   A2 Secondary

   A3 Early latent infection

   A4 Cardiovascular

   A5 Neurological

   A6 All other late and late latent stages

   A7 Congenital aged under 1 year

   A8 Congenital aged 1 year and over

2. New Cases of Gonorrhea

   B1 Post-pubertal infection

   B2 Pre-pubertal infection

   B3 Ophthalmia Neonatorum (includes CT and other STI pathogens)

3. New Cases of Other STIs

   C1 Chancroid

   C2 Lymphogranuloma venereum

   C3 Granuloma inguinale

   C4 Nongonococcal urethritis/cervicitis

   C5 Nonspecific genital infection with arthritis

   C6 Trichomoniasis

   C7 Candidiasis

   C8 Scabies

C9 Pubic lice (pediculosis pubis)

C10 Herpes Simplex (genital infections)

C11 Warts (condylomata acuminata/ human papilloma virus

C12 Molluscum contagiosum

C13 HIV/AIDS

C14 Hepatitis B or C infection

C15 HTLV-1

4. New Cases of Other Conditions

D1 Other treponemal infections e.g. yaws

D2 Other conditions requiring treatment in the centre e.g. MPox

D3 Other conditions not requiring treatment in the centre

* Jamaica's STI coding is historically based on the British Association for Sexual Health and HIV (BASH) which is no longer utilized. For ease of documentation, the Jamaican STI code will continue to be used.

STI Monthly Summary Statistics Report

The following is a copy of the STI monthly clinic report:

SEXUALLY TRANSMITTED INFECTION PROGRAMME

MONTHLY SUMMARY STATISTICS REPORT

Clinic/Site: **Month:** **Year:**

STI Condition		SEX	AGE GROUP												TOTAL
			0-4	5-9	10-14	15-19	20-24	25-29	30-34	35-39	40-44	45-49	50+	Unknown age	
Syphilis	Primary (A1)	M													0
		F													0
	Secondary (A2)	M													0
		F													0
	Early Latent (A3)	M													0
		F													0
	Late Latent	M													0
		F													0
	Tertiary	M													0
		F													0
	Congenital < 2 yrs	M													0
		F													0
	Congenital 2-4 yrs	M													0
		F													0
Genital Discharge Syndrome	Genital Discharge	M													0
		F													0
	Cervicitis														0
	Cervicitis	F													0
Genital Ulcer Disease	Genital Herpes	M													0

		F													0
	Chancroid	M													0
		F													0
	LGV	M													0
		F													0
Other STI and Non-STI Conditions	EPI Treatment for Syphilis (Partner)	M													0
		F													0
	EPI Treatment for GC/Chlamydia	M													0
		F													0
	Ophthalmia Neonatorum (1<month)	M													0
		F													0
	Epididimo-orchitis	M													0
		F													0
	Pelvic Inflammatory Disease (PID)	M													0
		F													0
	Genital Warts	M													0
		F													0
STI Clinic Data	# of STI Attendees	M													0
		F													0
	# of new STI Attendees	M													0
		F													0
	# STI Attendees Tested for HIV	M													0
		F													0
	# of STI Attendees Tested for Syphilis	M													0
		F													0
	# of STI Attendees Newly diagnosed for HIV	M													0
		F													0

	# of STI Attendees Dx Infectious Syphilis	M													0
		F													0
COMMENTS:															

STI Monthly Summary Statistics Report – An Explanation of Key Terms

Epi-treatment for Syphilis

- Indicate the number of contacts who received prophylactic treatment within the incubation period to prevent the progression of syphilis after exposure to an infectious case (primary, secondary, or early latent).
- Note: The contact must:
    - Have a negative syphilis serology upon initial examination
    - Have no clinical signs and symptoms suggestive of syphilis
    - Fit within the "critical period" for the stage of infectious syphilis

Epi-treatment for Gonorrhea/Chlamydia

- Indicate the number of contacts who received prophylactic treatment within the incubation period to prevent the progression of gonorrhea/chlamydia after exposure to a case of gonorrhea/chlamydia.

Number of STI Attendees

- The number of clients who attended the health facility for a STI. This is not limited to clients who attend STI-designated clinics only.

Number of New STI Attendees

- The number of clients who attended the health facility for the first time for an STI-related matter.

Number of STI Attendees tested for HIV

- Indicate the number of clients who attended a health facility for STI-related matters and were tested for HIV, including both new and returning clients.

Number of STI Attendees tested for Syphilis

- Indicate the number of clients who attended a health facility for STI-related matters and were tested for Syphilis, including both new and returning clients.

Number of STI Attendees Newly diagnosed with HIV

- Indicate the number of clients who attended a health facility for STI-related matters, tested positive for HIV and were never linked to care.

Number of STI Attendees diagnosed with infectious syphilis

- Indicate the number of clients who attended a health facility for STI-related matters and were diagnosed with primary, secondary and early latent syphilis.

STI Medical Record

Patients at STI clinics are assessed by a physician using the STI Medical Record, which informs CI reporting and aids in completing the STI Monthly Report.

Below is a sample of the updated STI Medical Record.

.

STI MEDICAL RECORD

MEDICAL RECORD #_____

DATE: _____/_____/_____ **dd mm yyyy**

D.O.B.: ____/____/____ **dd mm yy**

AGE: _____ yrs.

( ) New patient/First visit ( ) Follow-up visit
( ) Old patient/new episode __ ≤ 6 months __ >6 months
__ ≤ 12 months __ ≥12 months

1. NAME: _____ (Last) _____ (First) _____ (Middle) SEX : M ( ) F ( )

2. ADDRESS: _____ PARISH: _____ Tel: _____

Temperature: _____ Pulse / Heart rate: _____ Resp. rate: _____ Blood pressure: _____ Glucose: _____
Oxygen saturation: _____ Weight (kg): _____ Height (m): _____ BMI: _____

	Reason for visit	Duration
☐ Yes☐ No	Dysuria	
☐ Yes☐ No	Genital Discharge	
☐ Yes ☐No	Genital ulcer/sore	
☐ Yes ☐No	Rash	
☐ Yes ☐No	Itching	
☐ Yes ☐No	Abdominal pain	
☐ Yes ☐No	Contact to (Name STI)	
☐ Yes No	Other	

LAB TESTS ORDERED

☐ Blood ☐ TRUST/VDRL ☐ TPPA ☐HIV ☐Hepatitis B
☐ Hepatitis C ☐ Serum Creatinine ☐ Viral load
[]WetPrep:_____
[]Cultures:_____
[] PCR:_____
[]Other:_____

CLINICAL HISTORY

Partner with STI Complaint ☐Yes ☐ No
Complaint:_____
Hx of Chronic Illness: _____
Antibiotic (last two weeks) ☐Yes No_____
Allergy (esp. penicillin) ☐Yes ☐ No_____
STI last year ☐Yes ☐ No_____
Douching within the last month ☐Yes ☐No_____
Pap Smear ____/____/_____
dd mm yyyy

Drug screen: Have you used any of the substances below in the past 12months? Yes ☐ No ☐

Tobacco products	Yes ☐	No ☐
Marijuana	Yes ☐	No ☐
Crack cocaine	Yes ☐	No ☐
Use of Needles	Yes ☐	No ☐

Other_____

Depression screen: During the past month, are you bothered most of the time by…
1. Feeling down, depressed or hopeless? Yes ☐ No ☐
2. Having little interest or pleasure in doing things? Yes ☐ No ☐
If "Yes" to either, please proceed with Full Medical Profile and KADS-6 (adolescents)/PHQ-9 tool (adults)
Result:

SEXUAL HISTORY

❑ **Gender identity:** ☐ Male ☐ Female
❑ Other_____

Last sexual intercourse: ____/____/_____ dd mm yyyy

LMP:____/____/_____ dd mm yyyy

Gender of last sexual partner:
☐ Male ☐Female ☐ Other
_____
❑ Intimate Partner Violence
❑ Gender Based Violence

☐ HIV Status Sexual partner:
❑ Known
❑ Unknown

Number of partners: Last month ____ Last 3 months___ Last 12 months ____

In the past 12 months, gender of sex partners (all that apply): ☐Male ☐Female ☐ Other: _____

Condom use in last 3 months: ☐<1/2 time ☐>1/2 time ☐ Never

Anal sex in last 12 months: ☐ No ☐ Yes Anal Receptive: ☐ Yes ☐ No
Anal Insertive: ☐ Yes ☐ No Circumcised: ☐ Yes ☐ No
Breast Feeding: ☐ Yes ☐ No

In your lifetime, have you ever paid for sex? ☐Yes ☐ No

Is sex in exchange for money a source of income for you? ☐ Yes ☐ No ☐

Condoms used at last sexual encounter?	☐ Yes ☐ No
Condom at last anal sexual encounter?	☐Yes ☐ No
Previous HIV test?	☐ Yes ☐ No ☐ Don't know
HIV Status: ☐ Positive ☐ Negative ☐Unknown	If positive (tick all that apply): ☐ Not on ARV ☐ ART ≥6/12 ☐ ARV ≤6 /12

Contraception: ☐Pill ☐Inj ☐Condom ☐ TL ☐Other ☐None
Previous PrEP Use: ☐ Yes ☐ No

MOHW/HIV/STI Unit Revised January 2023

PHYSICAL FINDINGS Risk Score Females: _____

☐Yes ☐ No	Vaginal/urethral discharge.	☐ Normal - clear, mucoid. ☐ Abnormal - white, yellow/green, mod./profuse, curdy, frothy, odor, other_____
☐Yes ☐ No	Anal Health	☐ Normal ☐ Abnormal—ulcer, warts, discharge, blood, other _____
☐Yes ☐ No	Genital Ulcer _____	
☐Yes ☐ No	Rash _____	
☐Yes ☐ No	Warts _____	
☐Yes ☐ No	Cervix	☐ Normal ☐ Abnormal- erythema, erosion, polyps, ectropion, contact bleeding
☐Yes ☐ No	Cervical discharge	☐ Normal ☐ Abnormal – mucopurulent, bloody
☐Yes ☐ No	Lower abdominal tenderness _____	
☐Yes ☐ No	Cervical excitation tenderness _____	☐ Amine ☐ pH ____

OTHER: _____

Screening

General: (Please indicate the most current test dates for date requirements)
Fasting blood glucose : Date _____ ☐ Normal ☐ Abnormal Result: _____OGTT: Date _____ ☐ Normal ☐Abnormal Result: _____
Digital Rectal exam: ☐ Normal ☐ Abnormal (specify)_____ Pap Smear: Date_____ ☐ Normal ☐ Abnormal
Faecal Occult Blood: Test: Date : _____ ☐ Normal ☐ Abnormal
Clinical Breast exam: ☐ Normal ☐ Abnormal Mammography: Date_____ ☐ Normal ☐ Abnormal
PSA: Date_____ ☐ Normal ☐ Abnormal
HPV: Date_____ ☐Normal ☐ Abnormal Colonoscopy: Date : _____ ☐ Normal ☐ Abnormal

DIAGNOSIS AND TREATMENT

STI: ☐ No ☐ Yes Syndromic Dx: ☐ Genital discharge ☐ Genital ulcer ☐ PID ☐ Other Signs and Symptoms of Acute HIV _____
Clinical dx: _____ Therapy: _____ ☐ Given on site _____ ☐ Prescription
☐Azithromycin ____g PO STAT ☐Metronidazole ___ mg PO BID x ___days ☐Benzathine Penicillin G 2.4 MU: IM ___ weekly
☐Ceftriaxone 250 mg IM ☐ Doxycycline 100 mg PO BID x ___ days ☐Erythromycin 500 mg PO QID x ___ days
☐Ciprofloxacin 500 mg STAT ☐Norfloxacin 800 m STAT ☐Other _____

PrEP Initiation:
Medically ineligible to start PrEP: ☐ Yes ☐ No
Contraindications to TDF/FTC or 3TC/TDF
Tests Done: ☐ Yes ☐ No
Viral Load: ☐ Yes ☐ No
Hepatitis B: ☐ Positive ☐ Negative
Hepatitis C: ☐ Positive ☐ Negative
Serum Creatinine:_____
Willing to start PrEP: ☐ Yes ☐ No

To Start PrEP:
☐ HIV Negative
☐ Serum Creatinine > 60ml/min

PrEP prescribed at initial visit: ☐ Yes ☐ No
Regime: _____
Date of initiation:_____
Number of Months:_____

Health education: ☐Drug compliance ☐Disease awareness **Partner:** ☐Notification ☐Treatment ☐Reduction ☐ Adherence Counseling
☐ Condoms # issued_____Recommendation: _____
Clinician (signature): _____ Contact investigator (signature): _____
☐FNP: (print): _____ ☐MO: (print): _____
Referral to: ☐social worker ☐hospital ☐clinic ☐other _____ Return date: ___/____/_____
dd mm yyyy

Risk Score:

Partner has urethral discharge	2 ☐
New Partner in the last 3 months	1 ☐
>1 partner in the last 3 months	1 ☐
Not living with steady partner	1 ☐
Age <25 years	1 ☐
Total	____

[Risk Score ≥ 2, treat

Progress Notes and Additional Assessment:
_____
_____
_____
_____
_____
_____
_____
_____
_____

Contact Tracing

Contact tracing involves identifying and offering testing to contacts of individuals with infectious diseases. This process is guided by the Interview and Field Records previously outlined in this guide.

The specific steps of contact tracing carried out by the CI are as follows:

- Identification of the index case
- Obtain a list of sexual contacts and locating information
- Contact named contacts
- Screen all contacts identified
- Complete the interview and field records
- Record the outcome of testing, repeating the process for any positive individual identified.

To conduct safe and ethical contact tracing, the following steps must be included:

1. Obtain informed consent
2. Assess for Intimate Partner Violence (IPV)
3. Detect, monitor, report, and follow up on adverse events associated with index testing
4. Quality assurance and accountability assessment to identify gaps in index testing

Informed Consent

All contact investigators must ensure that index cases are properly registered at the health facility and that documented consent is recorded in the patient's file. The index testing and contact tracing process should then be clearly explained to the individual using a step-by-step approach.

Intimate Partner Violence Risk Assessment

Screening for Intimate Partner Violence (IPV) enables the contact investigator to determine the most appropriate partner notification strategy. If the screening indicates that notifying the partner could pose a risk to the patient’s safety, notification should be deferred until appropriate safety measures are in place.

The following questions may be used to screen clients for IPV. Before administering the questionnaire, reassure the client that the process is confidential and that these questions are routinely asked of all clients, as their safety is a priority. Use the name of each partner mentioned during the interview and ask the questions for each partner individually:

1. Has [partner’s name] ever threatened you with any form of violence or sexual abuse?
2. Has [partner’s name] ever hit, kicked, slapped, punched, or used a weapon to harm you?
3. Has [partner’s name] ever forced you to engage in any sexual activity that made you feel uncomfortable?

Monitoring and Reporting Adverse Events

Contact investigators must remain vigilant for any adverse events that may occur during the partner notification process. Once partner names are elicited and notifications are made, continued follow-up with both the index clients and their partners is required to monitor for any such events.

If an adverse event occurs, an **Incident Report Form** must be completed and submitted to the supervisory Contact Investigator, who will then forward it to the Medical Officer (Health). Each report must include:

- A clear description of the incident
- A plan of action
- Corrective steps to be taken
- A timeline for implementation and follow-up

Quality Assurance and Accountability

Each parish is required to hold bimonthly review meetings to assess the performance of the Contact Investigation programme, with a specific focus on index testing. The consistent use of Interview and Field Records helps maintain standardization across processes. Regular review of these documents promotes accountability and helps identify gaps in partner notification, ultimately supporting continuous improvement in service delivery at the site level.

PRACTICAL CASE MANAGEMENT OF COMMON STI SYNDROMES

Once the clinician has completed the assessment, the case will be classified and documented, and appropriate treatment will be provided according to established MOHW's algorithms. The tables below detail common STI-related signs and symptoms, along with common STI syndromes.

Common STI Signs and Symptoms	
Men	**Women**
Urethral discharge	Vaginal Discharge
Pain on Urination	Pain on urination
Scrotal pain/Swelling	Lower abdominal pain
Rashes (generalized/localized)	Rashes (generalized/localized)
Swollen glands (generalized/localized)	Swollen glands (generalized/localized)
Sores	Sores
	Menstrual abnormalities

Important STI Syndromes	
(I) Urethral discharge	(vii) Congenital syphilis
(II) Vaginal discharge	(viii) Anal discharge
(III) Lower abdominal pain – PID	(ix) Scrotal pain/swelling
(IV) Genital/anal ulcers – GUD (with or without adenopathy)	(x) Balanoposthitis
	(xi) Enteritis (recurrent/chronic)
(V) HIV/AIDS or related conditions	(xii) Acute Arthritis
(VI) Ophthalmia Neonatorium	(xiii) Genital/skin rashes

STI Management Algorithms

The following are algorithms developed to guide the management of the STI Client.

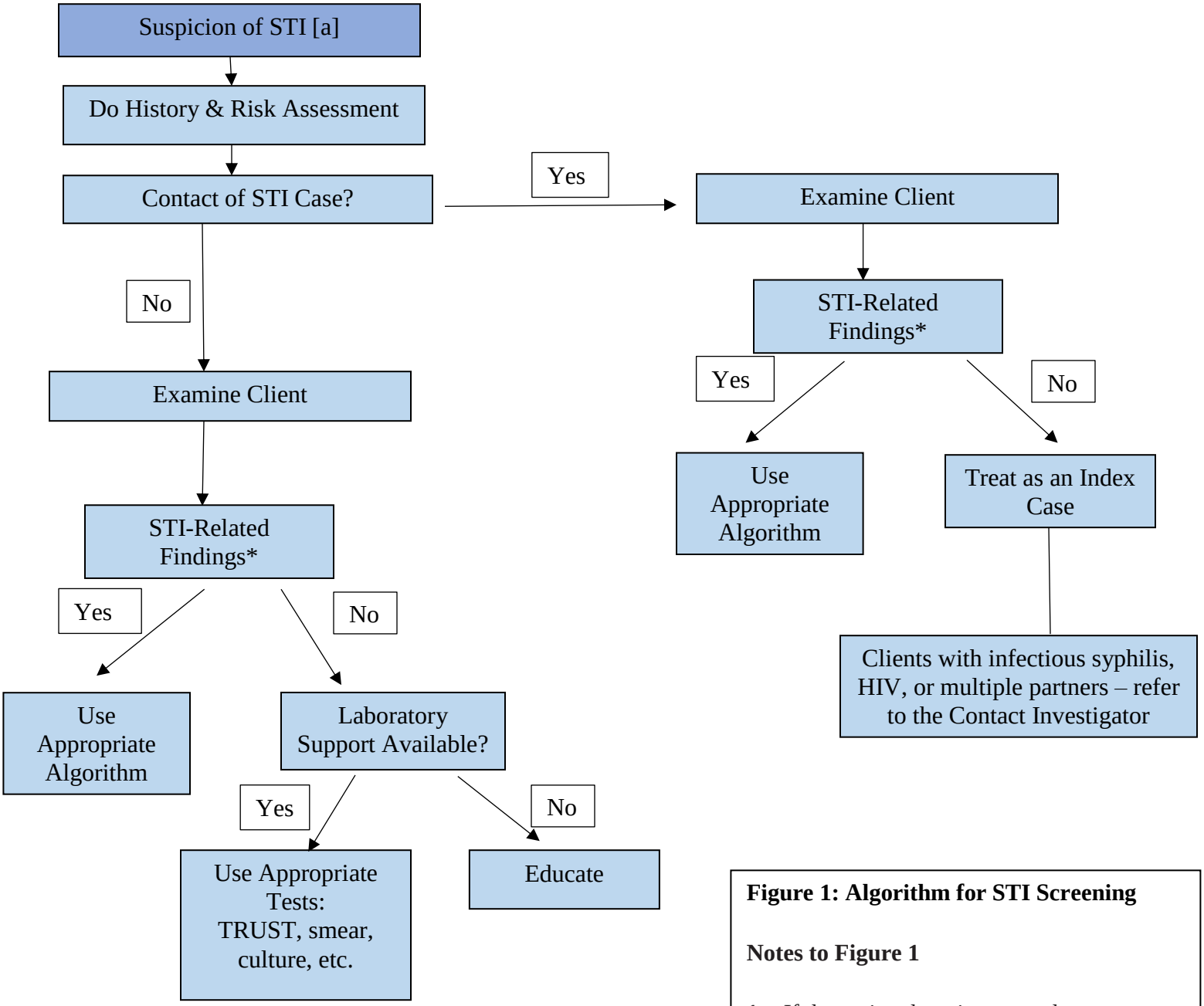


Figure 1: Algorithm for STI Screening

Notes to Figure 1

1. If the patient has signs, use the appropriate algorithm
2. Appropriate explanation should be given as to why (s)he needs treatment

Management of Urethral Discharge

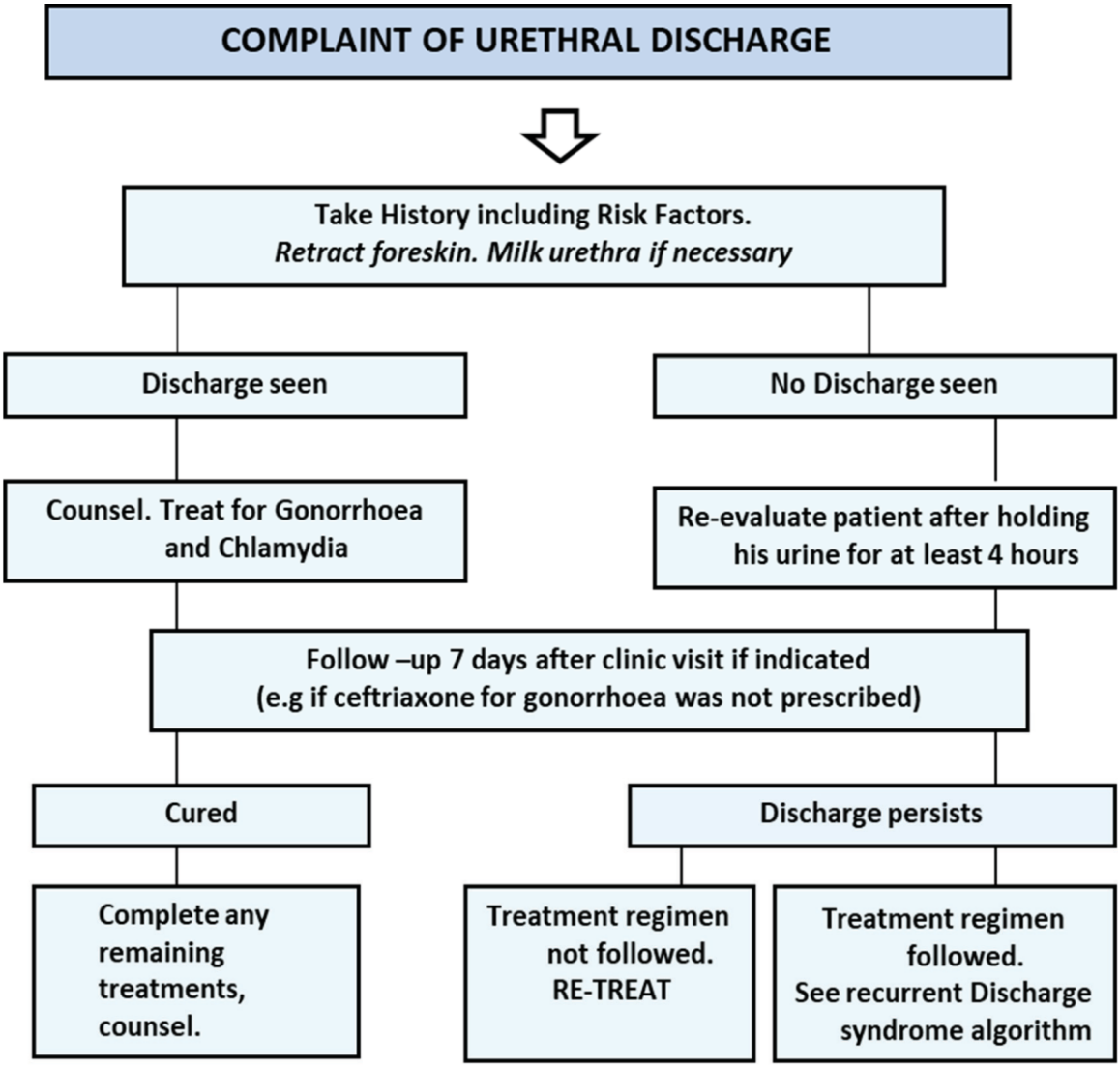


Figure 2: Algorithm for the Management of Urethral Discharge

Notes to Figure 2:

1. Early morning bead of pus; urinary "threads" in first urine passed; WBCs>4/HPF on Gram stained smear - suggests chlamydial/non-gonococcal urethritis.

2. Refer to urologist or hospital, especially patients > 35 years: non-STI causes of urethritis may be present.

Management of Testicular Pain

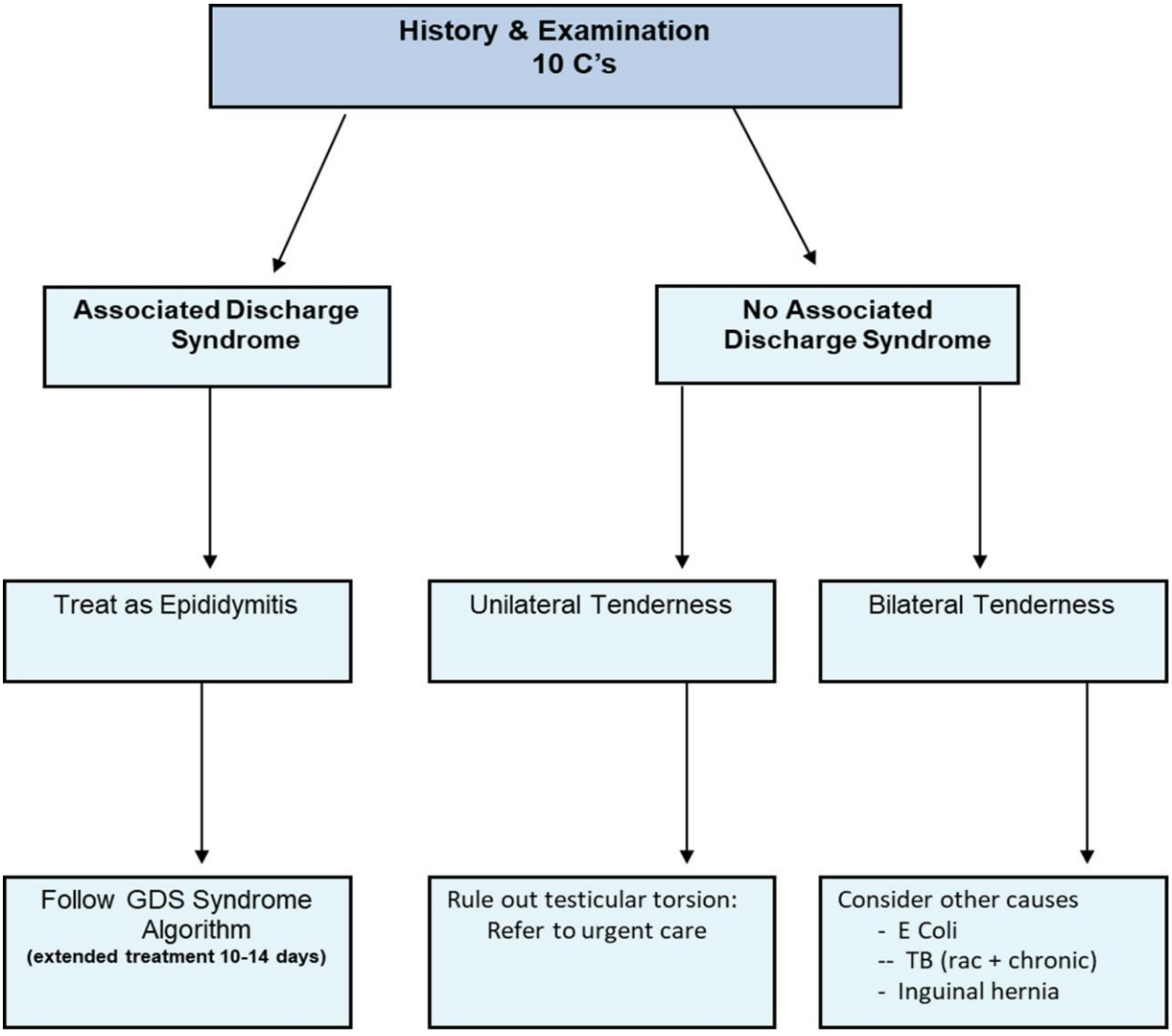


Figure 3: Algorithm for the Management of Testicular Pain

Notes to Figure 3:

The 10 C's are: comfort, confidence, confidentiality, communicate effectively, counsel, compliance, contacts, condoms, consideration of options for diagnosis and continued follow-up

Management of Persistent Discharge

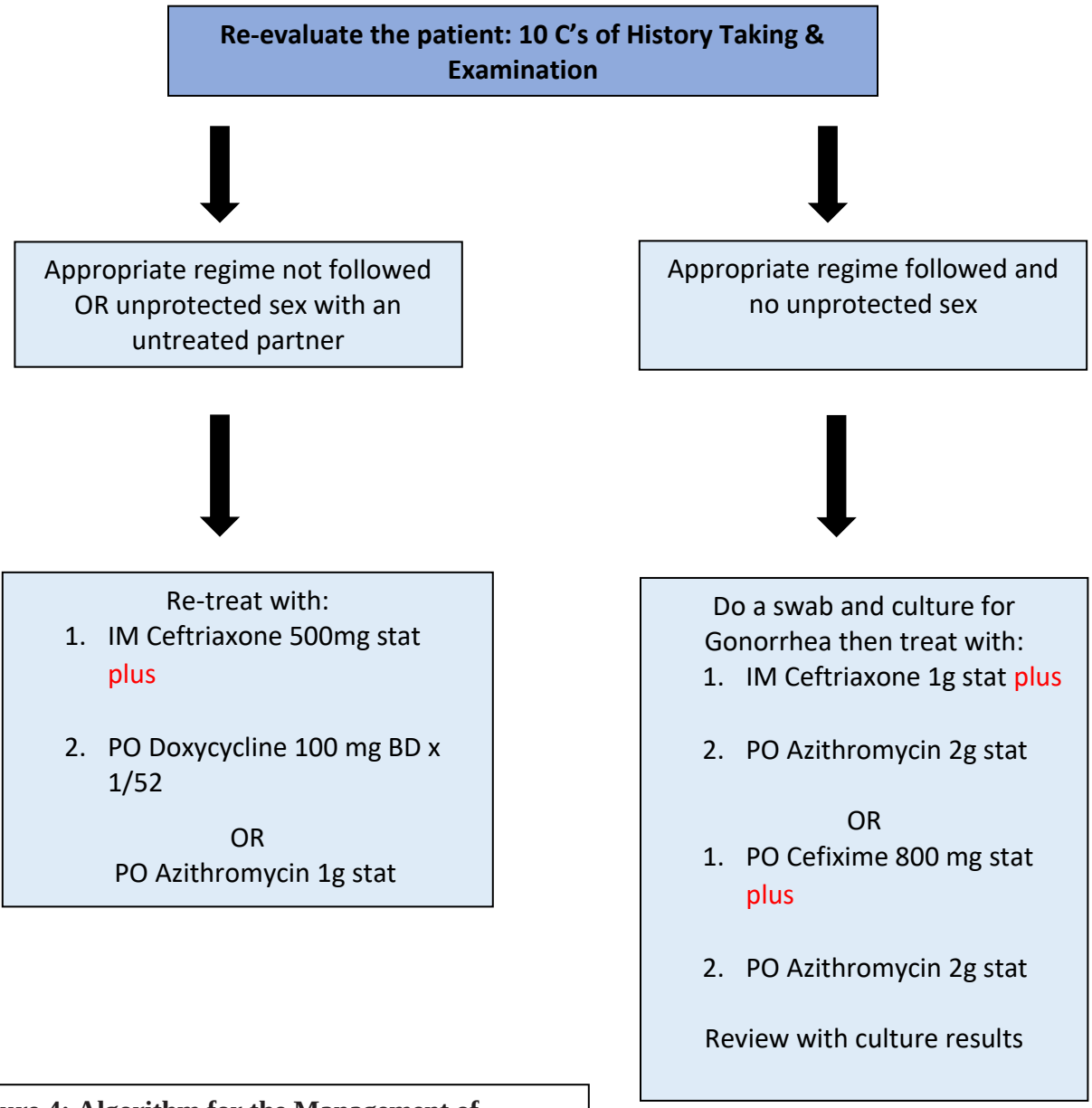


Figure 4: Algorithm for the Management of Persistent Discharge

Notes to Figure 4:

1. Abstain from all sexual activity OR use a condom with ALL types of sexual activity for 7 days OR until completion of course of medication to prevent spread to partner(s).

Management of Vaginal Discharge

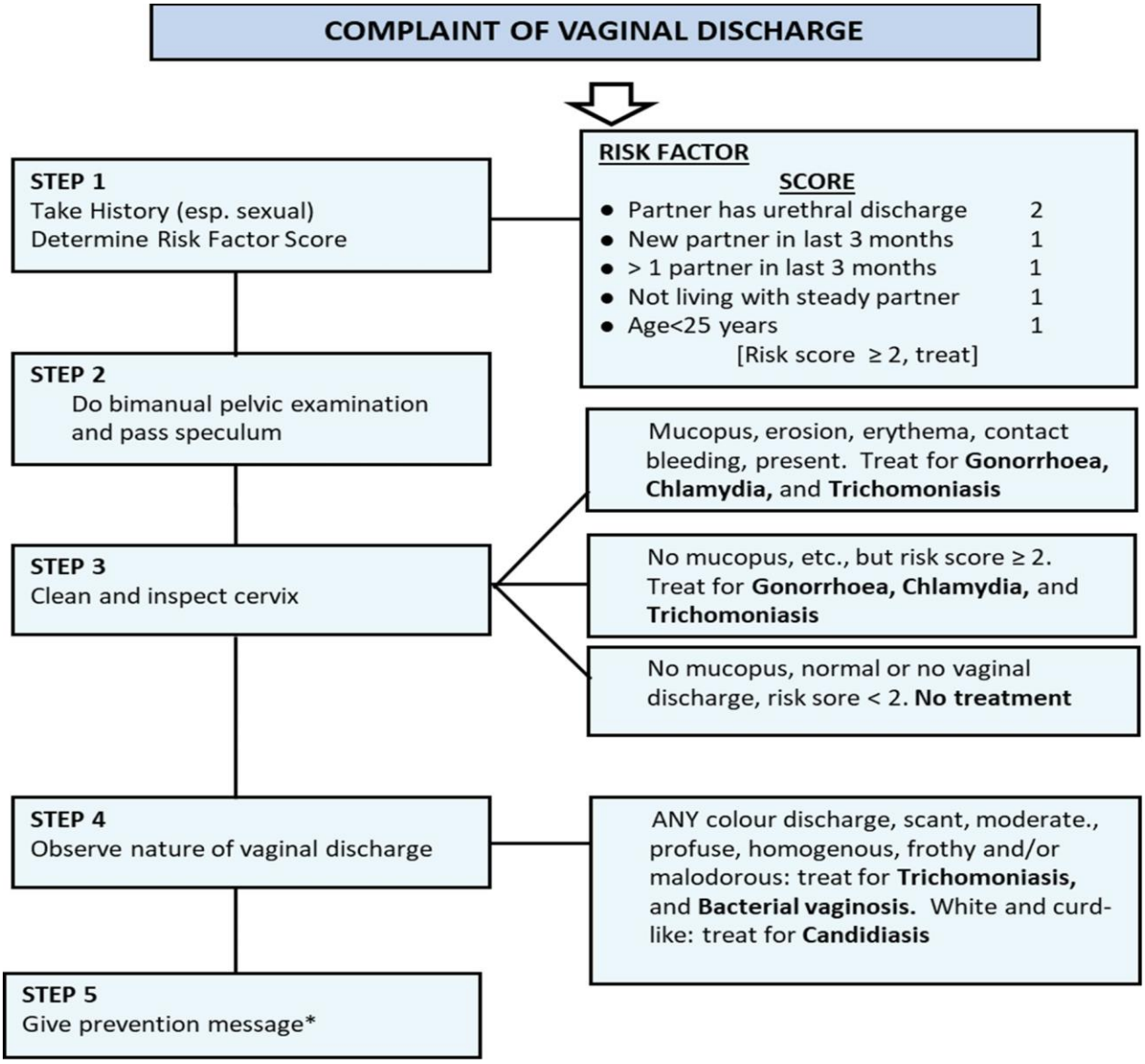


Figure 5: Algorithm for the Management of Vaginal Discharge

Notes to Figure 5:

*Comply with medication; condoms; contacts, counsel re risk reduction.

[Screen all patients for HIV/Syphilis. Do a pap smear and refer persistent cases of discharge to the gynaecologist].

Management of Anal Discharge

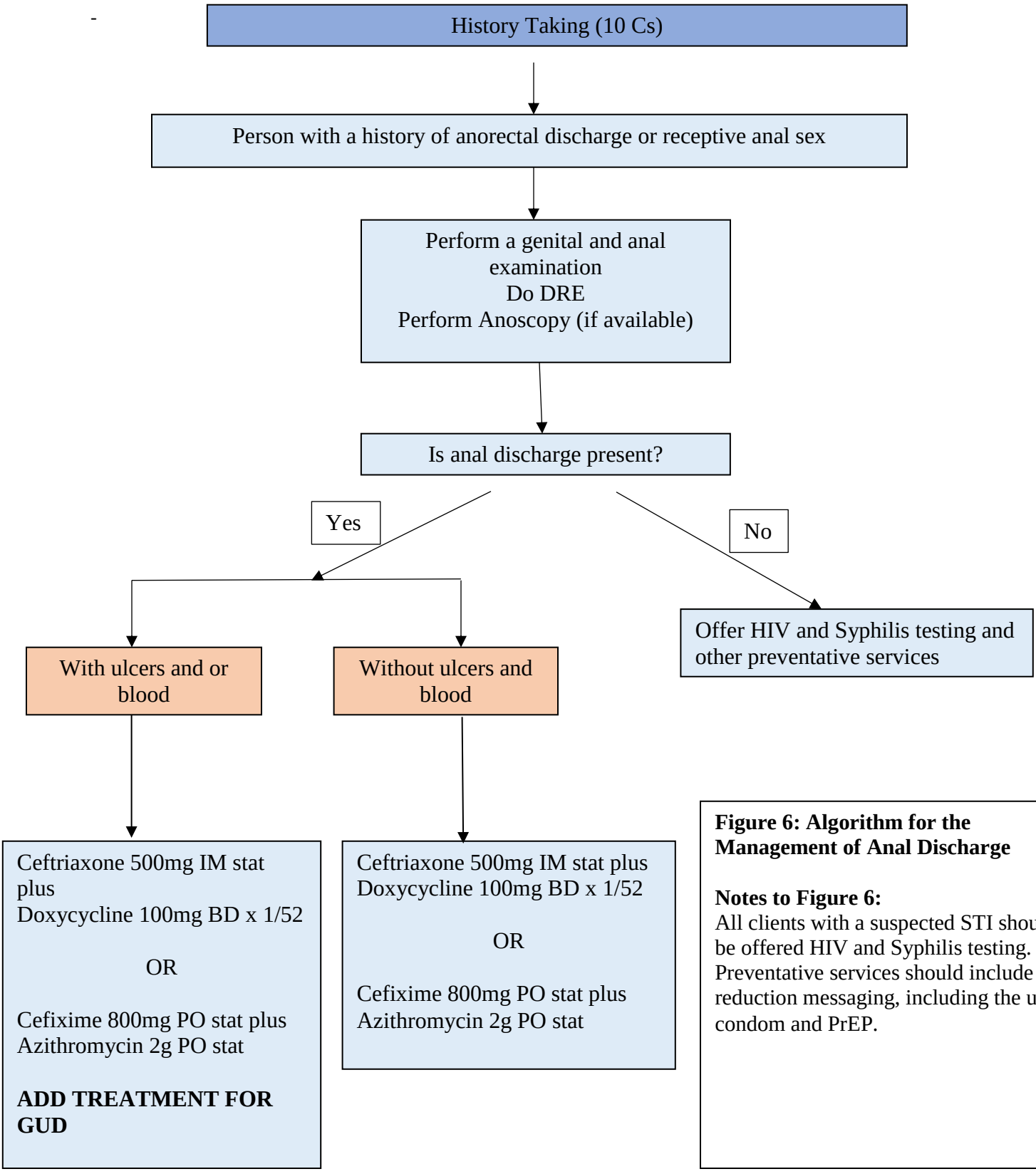


Figure 6: Algorithm for the Management of Anal Discharge

Notes to Figure 6:
All clients with a suspected STI should be offered HIV and Syphilis testing. Preventative services should include risk reduction messaging, including the use of condom and PrEP.

Management of Lower Abdominal Pain/Pelvic Inflammatory Disease

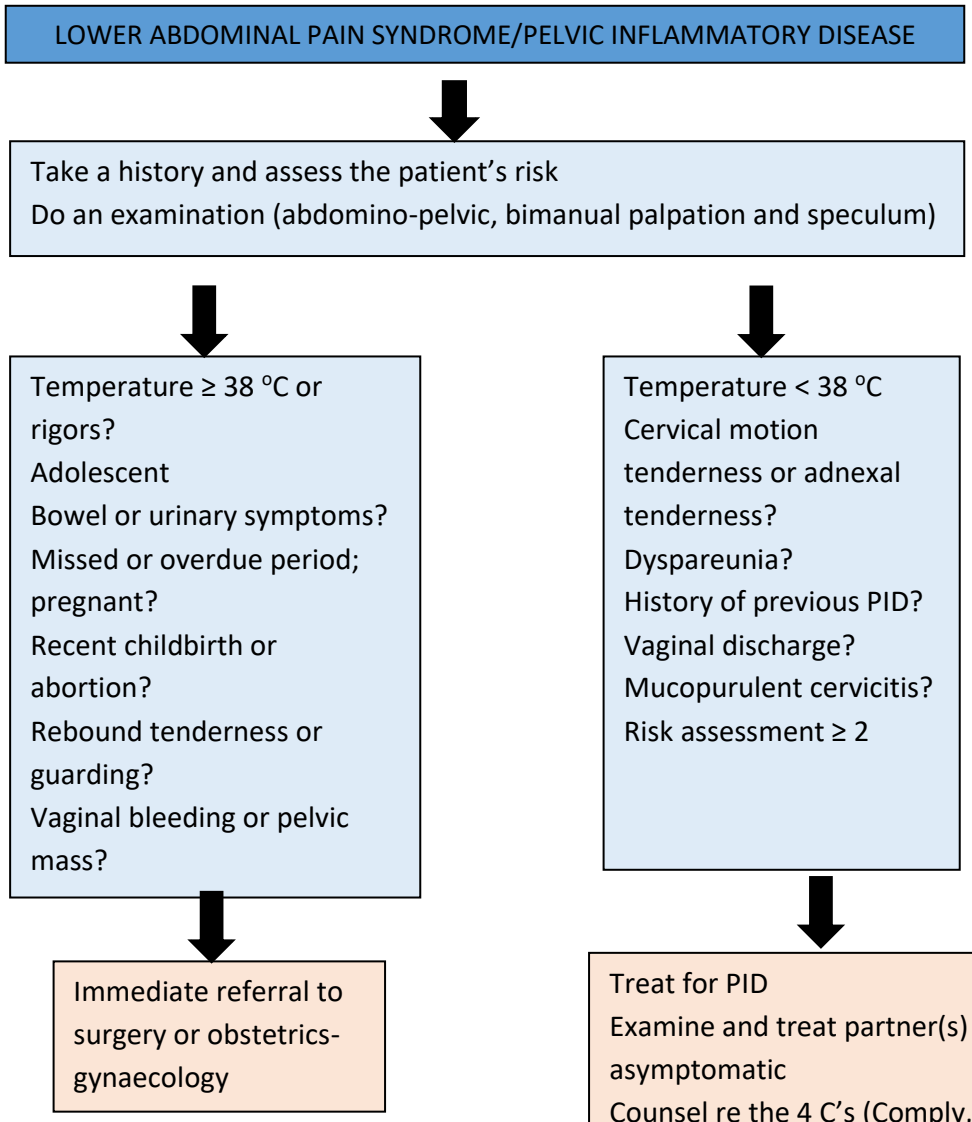


Figure 7: Algorithm for the Management of Lower Abdominal Pain/PID

Notes to Figure 7:

All patients should be tested for HIV and syphilis. A pap smear should be offered if none was done the within past year

Management of Genital Ulcer Syndrome

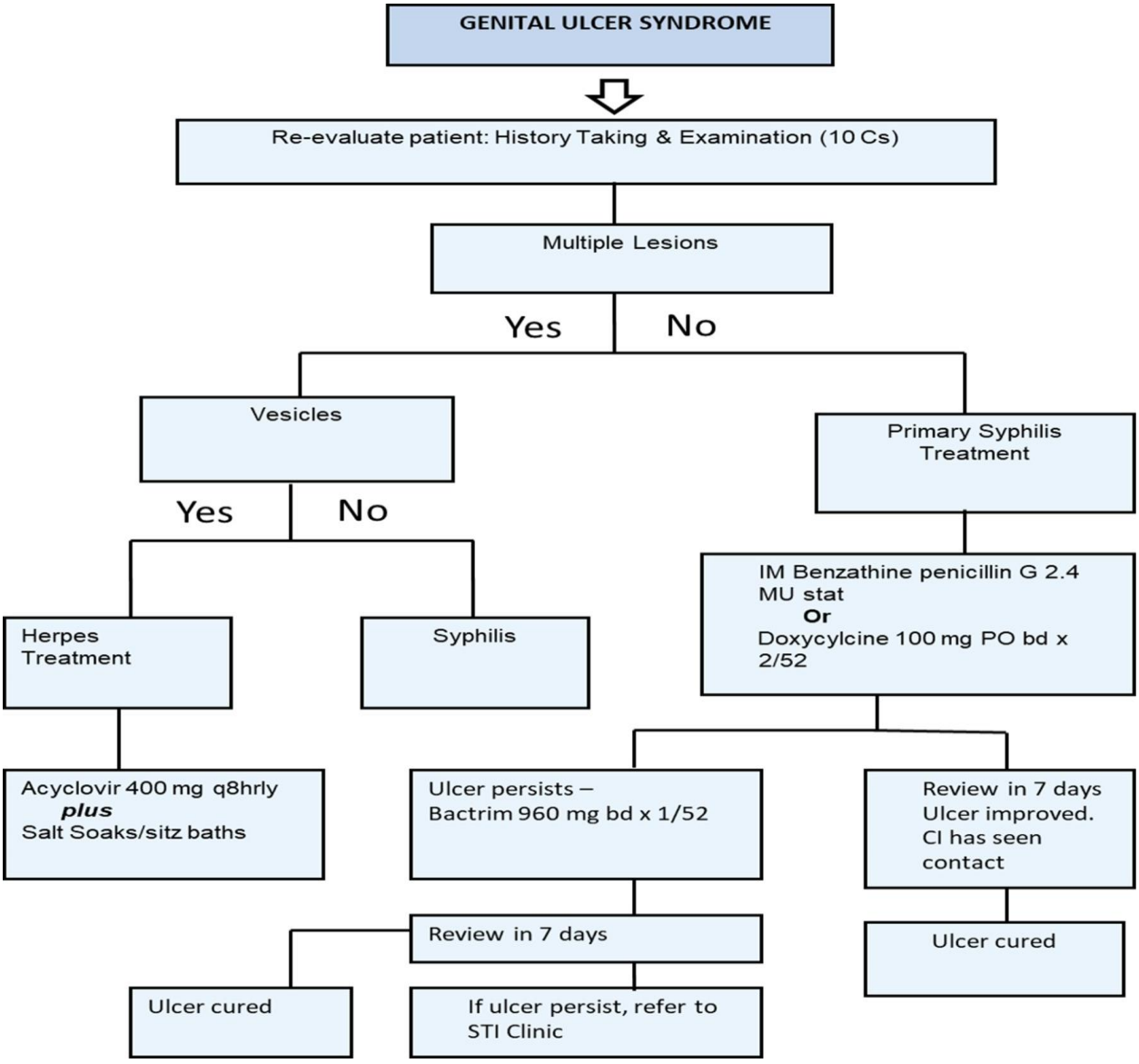


Figure 8: Algorithm of Management of Genital Ulcer Disease (GUD)

Notes for Figure 8:

Patients are to be advised not to use bleach, disinfectants or other chemicals to the genitalia to cure the lesions.

Management of Human T Lymphotrophic Virus Type I/II

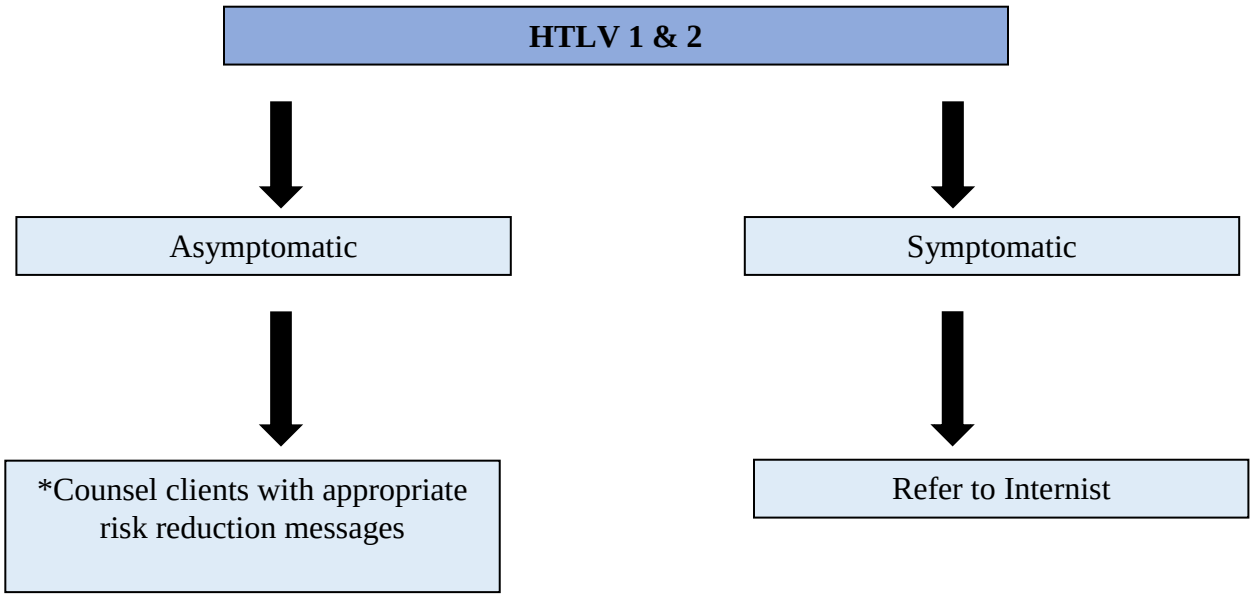


Figure 9: Algorithm for the Management of the Human T Lymphotrophic Virus type I/II (HTLV I/II)

Notes to Figure 9:

Clients who test positive for HTLV I or II should be encouraged to use condoms during sexual activity. Pregnant women with HTLV should also be advised to avoid breastfeeding.

SURVEILLANCE GUIDELINES FOR DIAGNOSIS, MANAGEMENT AND REPORTING OF NEW SYPHILIS CASES AND OTHER STI SYNDROMES

Syphilis

Syphilis, caused by the bacterium *Treponema pallidum*, progresses through distinct stages, with its infectiousness varying by stage. Infectious syphilis, lasting up to two years, includes the primary, secondary, and early latent stages. In the late latent stage, which usually occurs after two years of infection, the risk of transmission gradually decreases.

Infectious Syphilis - Duration up to two (2) years

A. **Primary Syphilis** - [A1]

Primary syphilis is characterized by the chancre - a hard, painless, well-defined papule or sore which may be single or multiple and usually appears 9 to 90 days (average of 21 days) after exposure to an infectious case. Lymph glands, when present are localized, firm, painless, and freely mobile.

Definition for Diagnosis

1. Sexual activity within the last three (3) months.
2. Presence of risk factors for STIs, e.g., multiple sex partners, a new partner within the last three (3) months, sex with a commercial sex worker, a partner with lesions, a history of having a previous STI, being under 25 years of age, or drug misuse.
3. Presence or history of a chancre, and/or a hard, painless, mobile inguinal lymphadenopathy.
4. Reactive serological test for syphilis (STS). This may be nonreactive in early infection.

Management

1. STS - Treponemal (SD Bioline) and a non-treponemal (TRUST) test. Dark field microscopy - A positive dark field microscopy is useful, but not a requirement (see page 80).
2. STS titre is usually < 16 dilutions (< R1:32)
3. N.B. Both treponemal and non-treponemal tests are negative in the early phase of primary syphilis and takes 1-4 weeks after the chancre appears to become reactive, i.e., 4 – 6 weeks after infection.
4. Treat with BPG 2.4 MU intramuscularly in a single dose.

5. Contact investigation and counsel (the 4 Cs - Compliance with medication, contact referral, condom use, and counsel for risk reduction; i.e., number of sex partners).

6. Epi-treatment for uninfected contacts (see page 85).

7. Follow-up with exam and STS 3-monthly x 4

B. **Secondary Syphilis** - [A2]

Secondary syphilis is characterized by symmetrical rashes that are typically non-itchy and vary in shape, size, and type (e.g., macular, papular, squamous, follicular, serpiginous, etc.). These rashes commonly appear on the palms and soles as well as the lower and upper face. These rashes may also present as condylomata lata in warm, moist areas of the body, especially the anogenital region. Generalized painless lymphadenopathy may also be present, along with systemic symptoms such as fever, headache, and malaise.

Definition for Diagnosis

1. Sexual activity within the last six (6) months.

2. Presence of risk factors for STIs, e.g., multiple sex partners, a new partner within the last three (3) months, sex with a commercial sex worker, a partner with lesions, a history of having a previous STI, being under 25 years of age, or drug misuse.

3. May have a history of a hard painless sore, and or, a hard, painless, mobile inguinal lymphadenopathy.

4. Presence of rashes (classic palmar/plantar presentation), condylomata lata, mucous patches, alopecia, generalized lymphadenopathy, or other manifestations.

5. Reactive STS.

Management

1. STS - Treponemal (SD Bioline) and a non-treponemal (TRUST) test. Darkfield microscopy - if moist lesions are present

2. STS is usually > 16 dilutions (> R1:32)

3. A negative treponemal test at this stage can be reasonably used to rule out syphilis.

4. Treat with BPG 2.4 MU intramuscularly in a single dose.

5. Contact investigation and counsel (the 4 Cs).

6. Epi-treatment for uninfected contacts (see epi-treatment for syphilis on page 85).

7. Follow-up with exam and STS 3-monthly x 4, then six-monthly x 2.

C. **Early Latent Syphilis** - [A3]

Early latent syphilis, as the name suggests, occurs after the lesions of primary and secondary syphilis resolve, marking the onset of a latent phase. During this stage, there are no clinical manifestations, and the infection's duration is less than or equal to two years.

Definition for Diagnosis

1. No clinical signs or symptoms of syphilis.
2. STS is currently reactive and the TRUST/VDRL titre is usually ≥ 1 in 4 dilutions (≥ R 1:8) [Strictly defined as having a documented non-reactive result within the last 12 months].
3. Sexual activity within the last 12 months [especially exposure to known A1 or A2].
4. Presence of risk factors for STIs, e.g., multiple sex partners, a new partner within the last three (3) months, sex with a commercial sex worker, a partner with lesions, a history of having a previous STI, being under 25 years of age, or drug misuse.
5. May give a history of a hard painless sore; or hard, painless, mobile inguinal lymphadenopathy; or generalised lymphadenopathy; or rashes, condylomata lata, mucous patches, alopecia etc.

Management

1. Non-treponemal and confirmatory treponemal test.
2. Treat with BPG 2.4 MU intramuscularly in a single dose.
3. Contact investigation and counsel (Remember the 4 Cs – Comply, Counsel, Condom & Contact).
4. Investigate contacts. If infected, treat according to stage.
5. Follow-up with exam and STS 6-monthly x 4, then annually until seroconversion or serofastness (no significant change in titre between 2 tests one year apart) is achieved.

Late or Non-Infectious Syphilis – Duration over two (2) years

> *During the first two years (primary, secondary, and early latent) syphilis is infectious to the sex partner and highly transmissible to the fetus during pregnancy*

D. **Late Latent Syphilis**

Late latent syphilis is an infection with syphilis greater than two (2) years duration without clinical manifestations. In late latency, the individual with untreated syphilis is less and less likely to transmit the infection to a sex partner or fetus during pregnancy as the latency progresses.

Late or Non-Infectious Syphilis – Duration over two (2) years

a. Cardiovascular syphilis - [A4]

Definition for Diagnosis based on Physician Consultation

1. Duration of infection for five (5) or more years.
2. STS is usually reactive and the titre is usually less than four (4) dilutions.
3. Damage to arteries and heart.

b. Neurological Syphilis - NSS [A5]

Definition for Diagnosis based on Physician Consultation

1. Duration of infection for three (3) or more years.
2. STS is usually reactive, and the titre is usually low.
3. Damage to the brain and nervous system.
4. NB. Infectious syphilis may occasionally present as acute neurosyphilis, e.g., meningitis, optic neuritis, etc.

c. All Other Late Latent, and Tertiary Syphilis - [A6]

Definition for Diagnosis based on Physician Consultation

1. Usually has no signs or symptoms of syphilis.
2. If symptoms of late syphilis exist, it may include gummas of skin, liver, heart, brain, spinal cord, and other visceral organs (tertiary signs)
3. STS is usually low but may be extremely high if visceral organs such as the liver are involved.

Management of Late or Non-infectious Syphilis

1. Non-treponemal and confirmatory treponemal test.
2. Treat with intramuscular BPG 2.4 MU once weekly for three (3) consecutive weeks.
3. Contact investigation is not normally required.
4. Refer to secondary/tertiary care for other investigations/treatments as indicated.

E. **Congenital Syphilis**

[Early; A7 = < 1 year and Late; A8 >2 Years]

Diagnosis and Management

1. See the section in the "Practical Case Management of Common STI Syndromes" but refer all suspected cases to a physician or hospital for diagnosis and treatment.
2. Investigate the mother and her contact(s). See below.
3. Investigate and record all cases occurring both at the clinic and in the hospital.
4. Complete a "Congenital Syphilis Report form" for each case; as soon as the pregnant woman is identified, the form should be initiated and submitted in the usual manner.

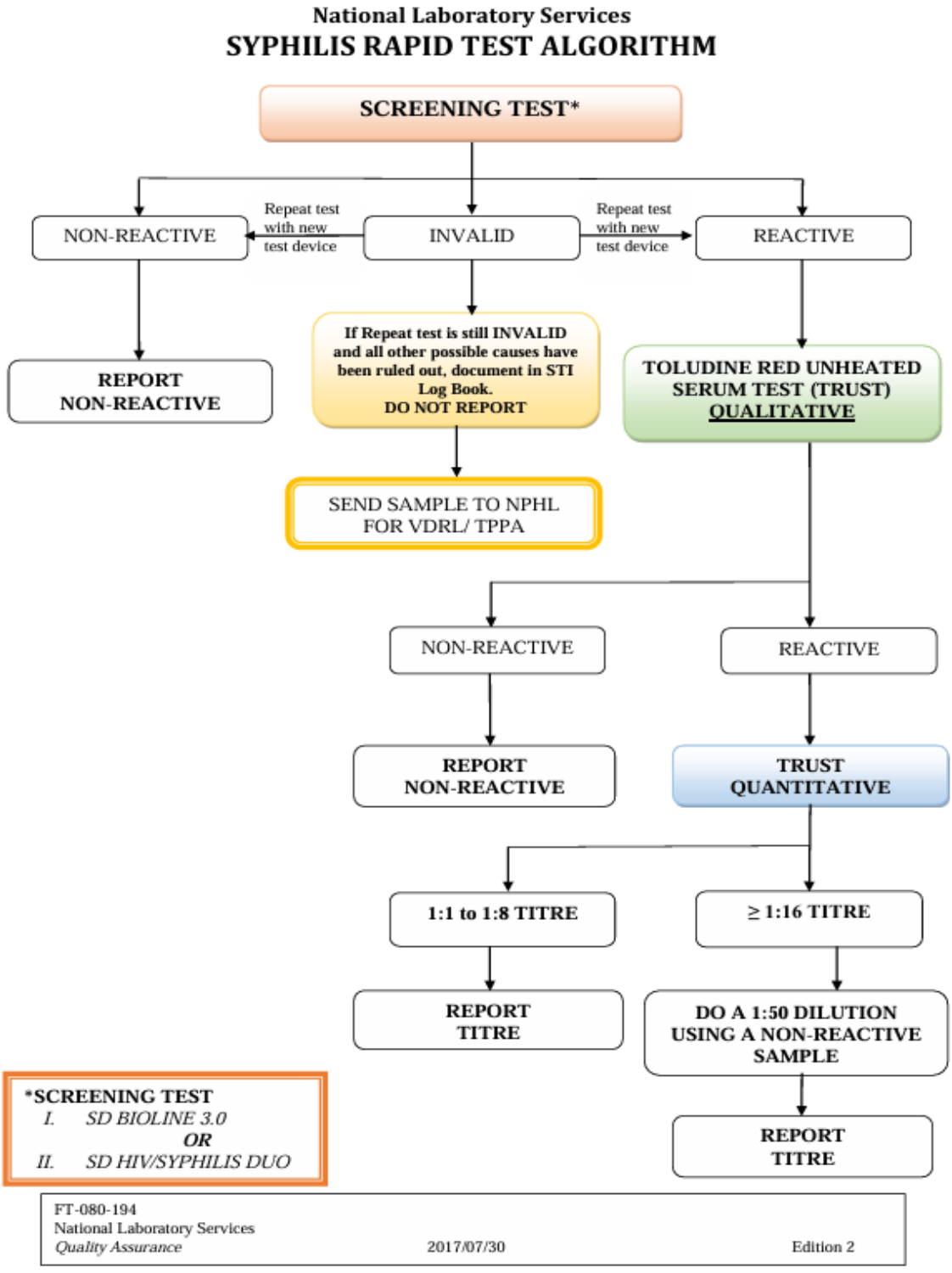
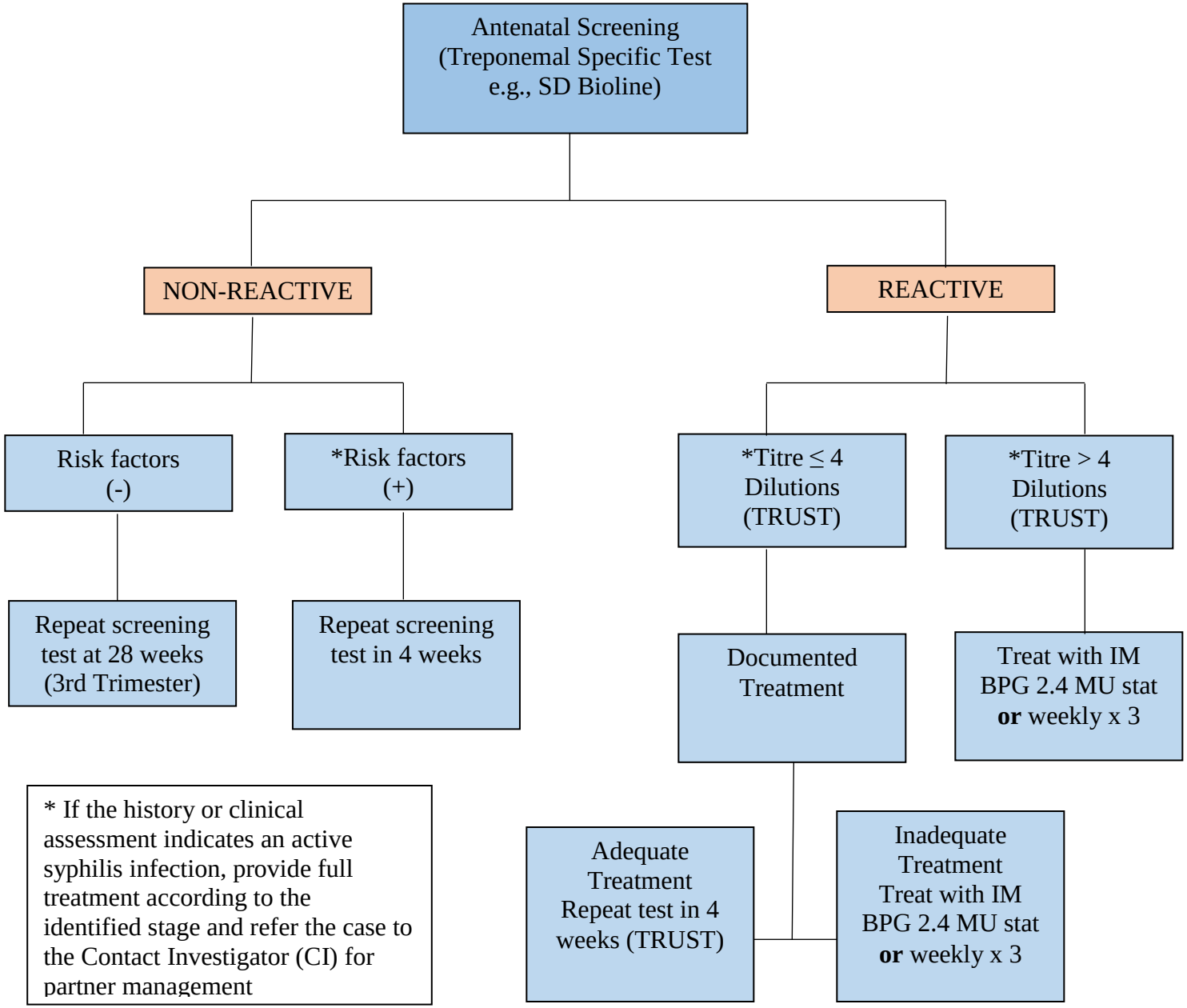


Figure 10: The National Laboratory Services Syphilis Testing Algorithm

Maternal Syphilis Algorithm



* If the history or clinical assessment indicates an active syphilis infection, provide full treatment according to the identified stage and refer the case to the Contact Investigator (CI) for partner management

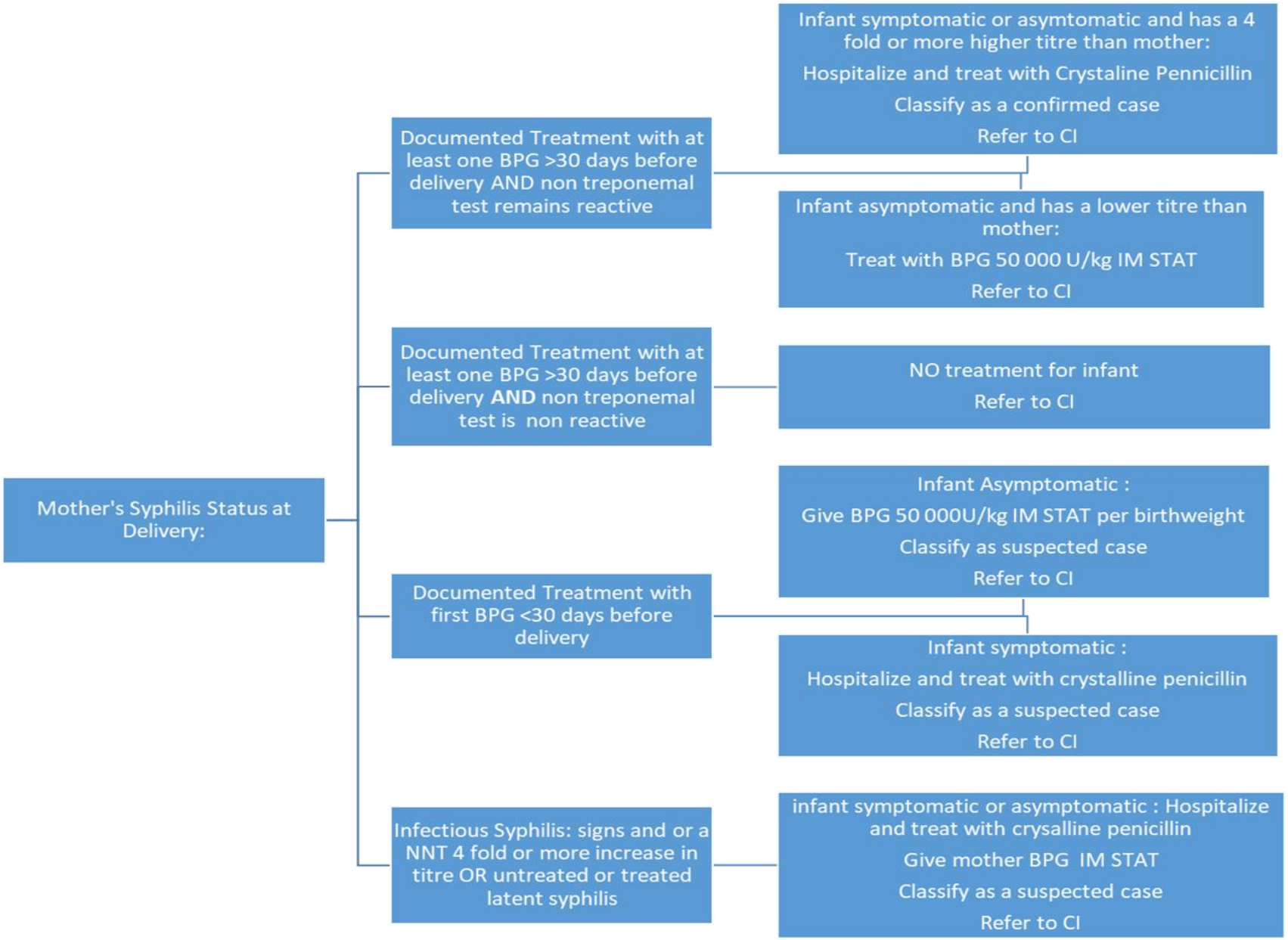


Figure 11: Algorithm for the Management of Congenital Syphilis

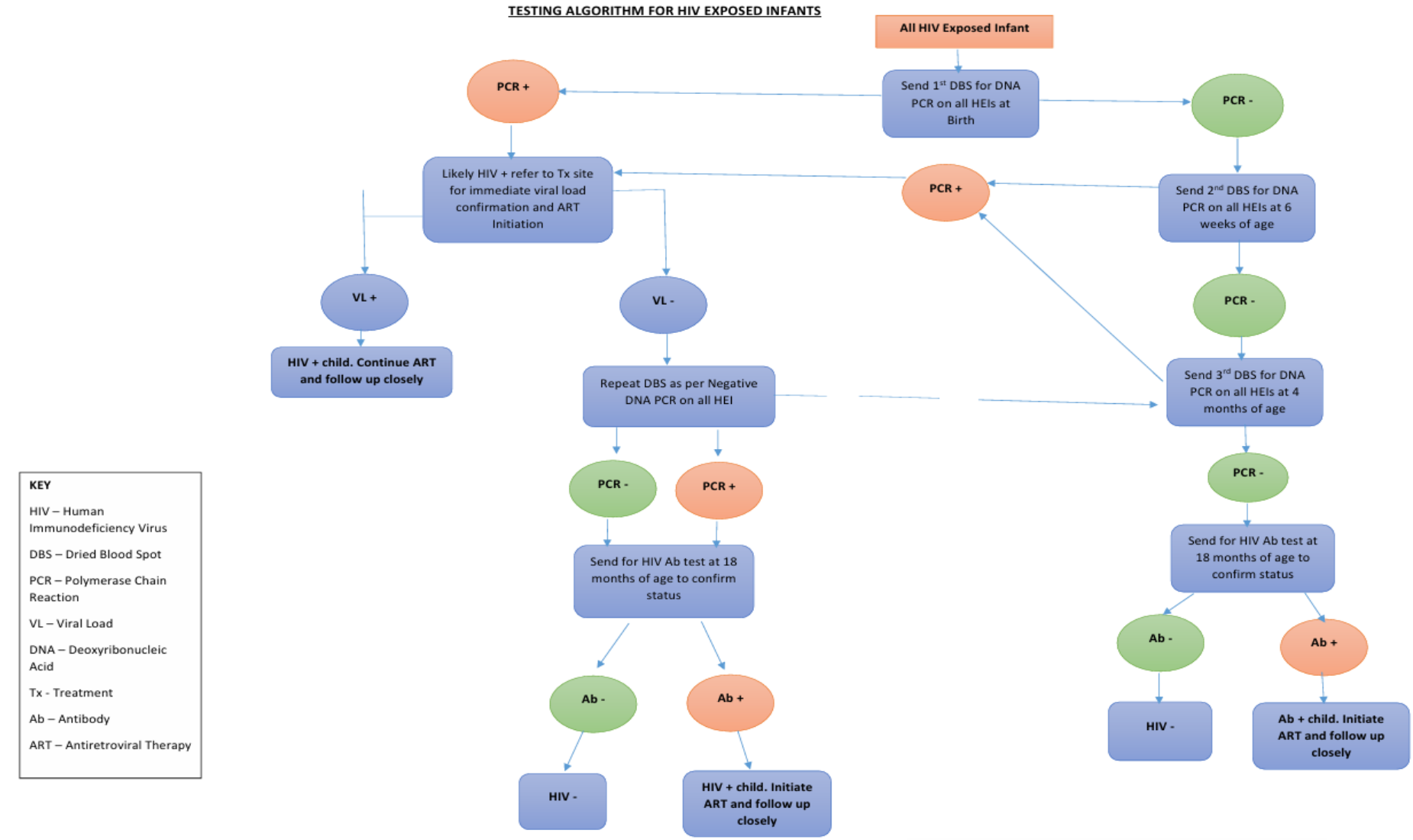


Figure 12: Testing Algorithm for the HIV Exposed Infant

Note for Figure 12:

All syphilis seropositive mothers should be educated and counselled for syphilis and HIV. A HIV screening test should also be offered

Other STI Syndromes

A. Genital Discharge Syndrome

Definition for Diagnosis

1. See the section in "Practical Case Management of Common STI Syndromes".

Record by age group all cases of abnormal urethral/anal discharge in men and cervico-vaginal/anal discharge in women. Following treatment, patients must be free from symptoms of their last episode of discharge for at least one month before they can be considered as new cases. This is to prevent duplicate reporting of follow-up cases of cervicitis/erosion. Record those women with/without cervico-vaginal discharge who also have cervicitis and/or erosion on speculum examination.

2. Trichomoniasis, Bacterial Vaginosis, and Candidiasis:

Record those women with cervico-vaginal discharge who, in addition, have either characteristic clinical findings (see the "Practical Case Management of Common STI Syndromes") and/or laboratory tests confirming these conditions.

B. Vaginal Discharge Syndrome

1. See the chart in "Practical Case Management of Common STI Syndromes" or its update.

C. Genital Ulcer Disease (GUD) Syndrome

Definition for Diagnosis

1. See the section in "Practical Case Management of Common STI Syndromes".

2. Record all cases of genital ulceration or bruising.

3. If there are either characteristic clinical findings (see "Practical Case Management of Common STI Syndromes") and/or laboratory evidence confirming an individual condition, e.g., genital herpes, syphilis, etc., list in the appropriate box. Otherwise, record under "Other".

Management of Genital Ulcer Syndrome

1. See the chart in "Practical Case Management of Common STI Syndromes" or its update.

2. In addition to syphilis testing, all patients diagnosed with an STI should be screened for HIV.

D. Other conditions

Record these cases using the appropriate forms:

1. Congenital Syphilis < 1 year (See page 87 - 88).

2. Ophthalmia Neonatorum (ON): See the case definition in "Practical Case Management of Common STI Syndromes". Cases should only be referred to the hospital if they are severe; otherwise, they should be evaluated by the clinic physician whenever possible. Remember that the male contacts of mothers with ON babies may be infected but asymptomatic. Give epidemiologic treatment to all recent male sexual contacts of the mother.

3. Pelvic Inflammatory Disease (PID) is a term used to describe the ascending spread of lower genital infection. It may involve the endometrium (endometritis), fallopian tubes (salpingitis), and pelvic peritoneum (peritonitis). See the section in "Practical Case Management of Common STI Syndromes". The presence of at least three of the following constitutes a diagnosis in a sexually active woman:

    a. Lower Abdominal Pain (LAP)

    b. Tenderness on moving the cervix [cervical excitation test (CET)]

    c. Adnexal tenderness with or without a mass

    d. Cervico-vaginal discharge

N.B., Epi-treatment: treatment given to a patient who is a known critical period contact for an infectious STI. Treatment is the same as that given to the index case.

Remember: Critical Period = Maximum incubation period for infection + duration of signs and symptoms (see page 39)

The contact must be clinically free from disease (no signs or symptoms) and serologically negative (in the case of syphilis)

1. All other STIs include Hepatitis B and or C, HTLV, CMV

2. Non-STI Referrals: Referrals made for routine blood tests and non-STI complaints.

GUIDELINES FOR THE INVESTIGATION OF CONGENITAL SYPHILIS, PAEDIATRIC HIV AND OPHTHALMIA NEONATORUM

Investigation of Congenital Syphilis (CS)

Congenital syphilis should be investigated as a priority using the form shown on pages 87 - 88. The following procedure should be used for investigation:

a) Test and treat the mother

b) Test and treat contact(s)

c) Refer to a physician for evaluation and follow-up the baby at monthly intervals

d) Report case(s) using the Congenital Syphilis Investigation Report Form, which should be initiated immediately upon identifying a reactive mother.

The reporting procedure should be carried out as follows:

a) Completed Congenital Syphilis Investigation Report forms should be submitted to the site supervisor weekly for review and subsequent submission to the Senior Contact Investigator (SCI).

b) Upon receiving the forms, the SCI should collate them, update the Congenital Syphilis Register, and inform the Medical Officer (Health).

c) The forms should then be forwarded for data entry.

Congenital Syphilis Report Form

CONGENITAL SYPHILIS INVESTIGATION REPORT

ANNEX 5: CONGENITAL SYPHILIS INVESTIGATION FORM

Parish	Date on Notification Form	Date Investigation assigned	Parish Code
INFANT INFORMATION			
Infant's Name	Age	Date of Birth	Gender M F
Name of Mother	Infant's Docket #	Health Centre/ Hospital name	
Telephone Number	Mother's Age	Home Address	
Mother's Docket #	Site of Delivery (Hosp/RMC/Home)		
CLINICAL DATA			
SYMPTOMS	Y N	SYMPTOMS	Y N
Generalized lymphadenopathy		Snuffles	
Vesiculo-bulious rash		Jaundice	
Pneumonia		Anaemia	
Neurological symptoms		Hepatosplenomegaly	
Mucous patches		Failure to thrive	
Other rashes		Was this a stillbirth?	
Was the birth premature?		Mother's VDRL Test (Result and Date)	

MOTHER'S INFORMATION				
# Children alive	# Stillbirths	# Miscarriages	# Lifetime sex partners	Interview Record #
ANC (this pregnancy) PRIVATE [ ] PUBLIC [ ] #Visits.....			VDRL/TRUST Test (Last pregnancy): [Y] [N] Result:..... Treatment [Y] [N]	
VDRL/TRUST Test (This pregnancy): [Y] [N] Result:..... Treatment [Y] [N]			Number and Date of doses of BPG	

MOTHER'S CONTACTS			
DISPOSITION	RESULTS	TYPE OF TREATMENT	DATE(S) OF TREATMENT
Baby's Father			
Other			

INVESTIGATION DATA			Treatment Given to Infant (with dates)
TEST	DATE	RESULT	
VDRL – Mother			
VDRL – Infant			
MHA-Tp – Infant			
CSF-VDRL			
Bone Xrays			DISPOSITION
Other			
COMMENTS			
FINAL CLASSIFICATION CONFIRMED CASE DISCARDED CASE			Signature: Date: Mo(H) Signature:

Infant Disposition Key:

1 = Treatment Completed

2 = Default (LFTU)/Incomplete Treatment

3 = Not Located/Not Treated)

All reports are to be submitted to

Principal Director,

The National Surveillance Unit

Ministry of Health and Wellness

15 Knutsford Boulevard,

Kingston 5

Investigation of Ophthalmia Neonatorum (ON)

Ophthalmia Neonatorum is characterized by a mucoid, mucopurulent, or purulent discharge from the eyes of newborns (≤1 month). Symptoms may include redness and swelling of the conjunctivae and palpebrae, with the latter potentially sticking together, resulting in the appearance of "sticky eyes." In severe cases, chemosis of the conjunctivae and pronounced edema and swelling of both eyelids may occur.

As for congenital syphilis, Ophthalmia Neonatorum should be investigated and reported using the Ophthalmia Neonatorum Investigation Report form shown on page 90. The procedure for investigation is as follows:

(a) Test and treat mother.

(b) Test and treat contact (s) if there is a history of disease in the contact(s).

(c) Refer to a physician for evaluation as necessary.

(d) Report on the special Ophthalmia Neonatorum Report Form.

The reporting procedure should be carried out as follows:

a) Ophthalmia Neonatorum is classified as a Class 1 notifiable disease; therefore, a Class 1 notification form must be completed and submitted to the Medical Officer (Health) within 24 hours.

b) The CI should complete the Ophthalmia Neonatorum Report forms and submit them to the site supervisor weekly for its review. These forms should then be forwarded to the Senior Contact Investigator (SCI).

c) Upon receiving the forms, the Senior Contact Investigator (SCI) should collate them and notify the Medical Officer (Health).

d) The forms should then be forwarded for data entry.

Ophthalmia Neonatorum Investigation Report

Appendix W: OPHTHALMIA NEONATORUM INVESTIGATION REPORT

Parish	Date on Notification Form	Date Investigation assigned	Parish Code
INFANT INFORMATION			
Infant's Name		Age / Date of Birth	Gender M F
Name of Mother		**Infant's Docket #**	Health Centre / Hospital name
Telephone Number	**Mother's Age**	Home Address	
Mother's Docket Number	Site of Delivery (Hosp/RMC/Home)		

CLINICAL DATA			
SYMPTOMS	Y N	SYMPTOMS	Y N
Muco-purulent or purulent conjunctivitis		Oedema and swelling of eyelids	
Redness of conjunctivae and palpebrae		Chemosis of conjunctivae	
Eyelids sticking together			
History of vaginal discharge in mother? Y N		Any Treatment given at birth?	
Treatment given at Home (i.e. home remedy etc)		Silver Nitrate Drops Y N Tetracycline Drops Y N	

MOTHER'S INFORMATION		
# Children alive	# Stillbirths	# Lifetime sex partners
# Miscarriages	ANC *(this pregnancy)* PRIVATE [ ] PUBLIC [ ] # VISITS	

LABORATORY DATA			TREATMENT
TEST	DATE	RESULT	
GRAM STAIN			
CULTURE			

COMMENTS

FINAL CLASSIFICATION	
CONFIRMED CASE DISCARDED CASE	Signature: Date: MO(H) Signature:

Investigation of Paediatric HIV Infection

Paediatric HIV is a priority A infection and should be investigated immediately using the Paediatric HIV Investigation Form shown on page 92. The investigation should be carried out as follows:

a. Counsel and test the mother

b. Counsel and test the contact(s)

c. Refer the mother and baby to a physician for evaluation and follow-up at monthly intervals. Prevention of mother-to-child transmission (PMTCT) is possible. Ensure that all HIV-positive mothers in your parish are educated about this intervention.

d. Report all case(s) on the Paediatric HIV Investigation Form

The reporting procedure should be carried out as follows:

a. The completed Paediatric HIV Report form(s) should be submitted to the site supervisor and Senior CI weekly for its review.

b. On receipt of said form(s), the SCI should collate them and notify the Medical Officer (Health).

c. The forms should then be forwarded for data entry.

Case Investigation Report – HIV-Exposed and Paediatric HIV Cases

National Surveillance Unit, National Epidemiology Branch,
ARC Building, 15 Knutsford Boulevard, 2nd Floor,
Kingston 5, St. Andrew, Jamaica
Email: surveillance@moh.gov.jm; Tel.: 876-633-7973

CASE INVESTIGATION REPORT

HIV-Exposed Infants and Paediatric Cases (Age Less Than 12 Years)

CONTACT TRACING	◯ Trace	◯ Do not trace

REASON FOR SUBMITTING CONFIDENTIAL REPORTING FORM		
◯ HIV-Exposed	◯ AIDS Diagnosis	◯ ART Initiation
◯ Initial HIV Diagnosis	◯ Death Notification	◯ Viral suppression
◯ Advanced HIV Diagnosis	◯ Treatment Regimen Change	◯ Risk History Update

INFANT'S/CHILD'S DEMOGRAPHICS	
First Name:	Middle Name(s):
Last Name:	Pet Name(s):
Country of Residence:	Parish (Jamaica):
House Number, Street Name, Community:	
Landmark or directions to address:	
Phone Number:	Email Address:
Sex Assigned at Birth: ◯ Male ◯ Female	**Infant's/Child's Medical Record Number:**
Date of Birth: *DD/MM/YYYY*	

RISK FACTORS			
Date of Assessment: *DD/MM/YYYY*	**Yes**	**No**	**Unknown**
◯ Infant born to HIV positive mother	◯	◯	◯
◯ Breastfeeding	◯	◯	◯
◯ Mother had High Viral Load in Pregnancy	◯	◯	◯
◯ Late HIV Diagnosis in Pregnant Mother	◯	◯	◯
◯ Non-Adherence to ART by Mother	◯	◯	◯
◯ Sexual Contact of Child	◯	◯	◯
◯ Lost to Follow-up Child	◯	◯	◯
◯ Lost to Follow-up Mother	◯	◯	◯
◯ Non-Disclosure of HIV Status by Mother to Partner/Relative	◯	◯	◯
◯ Vaginal Delivery	◯	◯	◯
◯ Infant/Child Received Transfusion of Blood Products	◯	◯	◯
◯ Other (specify):	◯	◯	◯

Nutrition: ◯ Exclusively Breastfed ◯ Replacement Feed Only ◯ Both Breastfed and Replacement Fed

CLINICAL STATUS (DIAGNOSIS BY PHYSICIAN)			
Date of Assessment: *DD/MM/YYYY*	**Yes**	**No**	**Unknown**
◯ Asymptomatic	◯	◯	◯
◯ Generalised lymphadenopathy	◯	◯	◯
◯ Generalised dermatitis	◯	◯	◯
◯ Unexplained persistent parotid enlargement	◯	◯	◯
◯ Hepatosplenomegaly	◯	◯	◯
◯ Recurrent diarrhoea	◯	◯	◯
◯ Recurrent/chronic upper respiratory tract infections	◯	◯	◯

CLINICAL STATUS (DIAGNOSIS BY PHYSICIAN)			
Date of Assessment: *DD/MM/YYYY*	Yes	No	Unknown
◯ Pulmonary tuberculosis	◯	◯	◯
◯ Failure to thrive	◯	◯	◯
◯ Multiple or recurrent bacterial infections	◯	◯	◯
◯ Neurological involvement	◯	◯	◯
◯ Oesophageal candidiasis	◯	◯	◯
◯ Recurrent lower respiratory tract infections	◯	◯	◯
◯ Cytomegalovirus retinitis	◯	◯	◯
◯ Extrapulmonary tuberculosis	◯	◯	◯
◯ Other (specify):	◯	◯	◯

Clinical Notes:

ANTIRETROVIRAL THERAPY (INFANT/CHILD)

ARV	Dosage	Actual Duration Received (weeks)	Comments
◯ Nevirapine			
◯ Zidovudine			
◯ Lamivudine			
◯ Abacavir			
◯ Lopinavir/Ritonavir			
◯ Dolutegravir			
◯ Tenofovir			
◯ Other (specify):			

Current Status of Patient:

◯ Pending	◯ Advanced HIV	◯ AIDS	◯ AIDS Death
◯ HIV	◯ HIV Negative		

Date of Onset of Symptoms: *DD/MM/YYYY*	Date diagnosed as Advanced HIV: *DD/MM/YYYY*
Date diagnosed as AIDS: *DD/MM/YYYY*	Date of death: *DD/MM/YYYY*

Cause of death:

i.	ii.	iii.
(immediate cause)	(intermediate cause)	(underlying cause)

MOTHER'S INFORMATION	
Mother's ID Type: ◯ Driver's License ◯ TRN ◯ Passport Number ◯ NIDS	Mother's ID Number:
Mother's First Name:	Mother's Middle Name(s):
Mother's Last Name:	Mother's Pet Name(s):
Mother's Maiden Name (if applicable):	
Mother's Date of Birth: *DD/MM/YYYY*	Mother's Age:
Mother's Country of Residence:	Parish (Jamaica):
House Number, Street Name, Community:	

Name of Patient:_____ **Medical Record Number:**_____

Landmark or directions to address:	
Phone Number:	Email Address:
Mother's Medical Record Number:	Treatment Site:
HIV Status of Mother: ◯ Positive ◯ Negative	Date of Mother's Confirmatory HIV Test: *DD/MM/YYYY*

Laboratory where Mother's Confirmatory HIV Test Performed:

◯ National Public Health Laboratory	◯ University Hospital of the West Indies	◯ Other Government Laboratory Facility	◯ Other

Site of Delivery:

◯ Public Hospital	◯ Private Doctor	◯ Health Centre	◯ **Other (Specify):**
◯ Private Hospital	◯ Private Obstetrician	◯ Home	

Gravida:	Parity:

Treatment During Pregnancy

Antenatal ARV Treatment	**ART Start Date (*DD/MM/YYYY*)**	**Antenatal ARV Treatment**	**ART Start Date (*DD/MM/YYYY*)**
◯ Tenofovir/Lamivudine		◯ Atazanavir/Ritonavir	
◯ Tenofovir/Emtricitabine		◯ Darunavir/Ritonavir	
◯ Abacavir/Lamivudine		◯ Zidovudine/Lamivudine	
◯ Dolutegravir		◯ Nevirapine	
◯ Lopinavir/Ritonavir		◯ Other (specify):	

Antenatal Care (this pregnancy):

◯ Public Hospital	◯ Private Doctor	◯ Health Centre
◯ Private Hospital	◯ Private Obstetrician	◯ None

Mother's Next of Kin

First Name:	Last Name:
Phone number:	Email address:
Address: Lot/ Street/Community/Parish:	

Relationship to the Mother:

◯ Husband	◯ Daughter	◯ Brother	◯ Aunt	◯ Friend
◯ Mother	◯ Son	◯ Grandmother	◯ Uncle	◯ Other
◯ Father	◯ Sister	◯ Grandfather	◯ Cousin	

FATHER'S INFORMATION

Father's First Name:	Father's Middle Name(s):
Father's Last Name:	Father's Pet Name(s):
Father's Date of Birth: *DD/MM/YYYY*	Father's Age:
HIV Status of Father ◯ Positive ◯ Negative	Date of Father's Confirmatory HIV Test: *DD/MM/YYYY*

Laboratory where Father's Confirmatory HIV Test Performed:

◯ National Public Health Laboratory	◯ University Hospital of the West Indies	◯ Other Government Laboratory Facility	◯ Other

SAMPLE AND LAB INFORMATION

HIV Testing for Infant/Child	Date (DD/MM/YYYY)	Result	Comment
◯ PCR at birth	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
◯ 2-Week PCR (if high risk)	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
◯ 6-Week PCR	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
◯ 4-Month PCR	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
◯ 18-Month ELISA	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
◯ Determine Rapid Antibody	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
◯ Uni-Gold Rapid Antibody	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
◯ Other_____	DD/MM/YYYY	◯ Positive ◯ Negative ◯ Indeterminate	
Comments:			

Notifier's Details		
Date of Reporting *(DD/MM/YYYY)*: /		
First Name:	Last Name	
Professional Group:		
Phone number:	Email Address:	
Name of Institution:		
Parish:	Community:	
Office Number/Street Name:		
Health Region: ◯ SERHA ◯ NERHA ◯ SRHA ◯ WRHA		
Name of Parish MO(H):	Signature of MO(H):	Date signed by MO(H): *DD/MM/YYYY*

Name of Patient:_____ **Medical Record Number:**_____

Definitions (Diagnosis by physician)

Asymptomatic	No symptoms or signs suggestive of HIV infection.
Hepatosplenomegaly	Enlargement of liver and/or spleen.
Generalised Lymphadenopathy	Enlargement of more than 1 cm of more than 2 noncontiguous lymph node groups. Common sites are head, neck, post- or pre- auricular, supraclavicular, axillary, epitrochlear, and inguinal.
Cytomegalovirus retinitis	Commonly asymptomatic; fundoscopy usually reveals bilateral changes, often affecting the macula ranging from white flecks giving a granular appearance to more fulminant disease affecting the full thickness of the retina with retinal oedema, retinal vascular sheathing, or haemorrhages. CMV can also affect other organ systems.
Extrapulmonary tuberculosis	Systemic illness that may include prolonged fever, night sweats, weight loss; presentation dependent on organ system involved; culture, microscopy or other laboratory evidence of Mycobacterium tuberculosis in specimen.
Failure to thrive	Weight for age and sex below 3rd to 5th percentile; loss of weight associated with crossing two or more percentiles on a standardised growth curve; weight for height z-score below minus 2 (-2).
Generalised dermatitis	Unexplained rash to multiple parts of the body.
Recurrent diarrhoea	Three (3) or more watery stools in a 24-hour period, recurring on two or more days during a 14-day period.
Unexplained persistent parotid enlargement	Unexplained enlargement of the parotid glands, usually bilateral and painless.
Recurrent/chronic upper respiratory tract infections	Two or more episodes of upper respiratory tract infection such as ear, nose, throat, sinuses within a six-month period.
Oesophageal candidiasis (or candidiasis of trachea, bronchi or lungs)	Candidiasis of the mouth in infants above 3 months of age; may be associated with decreased feeding or crying during feeding.
Neurological involvement	Unexplained recent onset of seizures, toxoplasmosis, cytomegalovirus (CMV), cryptococcus, encephalopathy. Delayed development and/or loss of developmental milestones.
Recurrent lower respiratory tract infections	Two or more episodes of lower respiratory tract infections within a 6-month period presenting with respiratory distress or positive findings on examination e.g., crackles, consolidation; and/or positive findings on imaging or laboratory investigation.
Multiple or recurrent bacterial infections	Two or more episodes of bacterial infection requiring hospitalisation within a 6-month period.
Pulmonary tuberculosis	Symptoms of chronic cough, fever, night sweats, weight loss; abnormal findings on imaging e.g., chest X-ray and/or demonstration of the Mycobacterium tuberculosis pathogen by microscopy, culture or molecular (e.g., polymerase chain reaction) methods.

Name of Patient:_____ **Medical Record Number:**_____

GUIDELINES FOR THE INVESTIGATION OF HUMAN IMMUNODEFICIENCY VIRUS (HIV)

Human Immunodeficiency Virus is mainly spread via sexual contact. Contact investigators should be aware of all confirmed cases of HIV infection to facilitate contact investigation and linkage to care. Persons who are notified that they have tested positive for HIV should be counselled regarding important facts about HIV. The following fact sheet and recommended counselling guidelines are provided to assist the contact investigator in this task.

Disease information

HIV is passed from person to person by close contact with infected blood, saliva, semen, and vaginal fluids. The most common way for HIV to spread is by sexual contact. Children born to HIV-positive mothers face a significantly higher risk of becoming infected.

After exposure to the virus, the virus may take hold and replicate using the body's white blood cells (CD4). Replication starts at the site of infection, and within days, spreads throughout the body. The virus is able to establish reservoirs in blood cells, lymph nodes, the brain and the intestinal tract.

This stage is referred to as the acute stage. During this time the body mounts a strong immune response referred to as "seroconversion". Antibodies are produced during this period and persons may experience flu-like symptoms as the only symptoms during this phase.

Six weeks after a period with no signs and symptoms denotes the beginning of the latency period. During this period there is a gradual increase of the viral load and a decrease of the CD4 cells, which may span approximately 10 years if no treatment is received.

Following this period without treatment, the HIV-positive patient will progress to end-stage disease or AIDS. This is when the CD4 falls below 200 cells per microliter and there is a rapid increase in the viral load. This depletion of the immune system results in multiple infections (opportunistic infections) and cancers. Of note, successful implementation of HIV medications prevents the progression to end-stage disease. For individuals diagnosed with AIDS, the initiation of medication can lead to significant improvement in managing the disease, resulting in a long and healthy life.

Counselling HIV-Positive Persons

The following steps should be taken when counselling persons who have been told that they are HIV positive:

1. **Confirm Results** - Be sure of the results of the test: HIV testing follows a specific algorithm which ensures that a report of a positive HIV case comes after testing and confirmation.
2. **Assess Knowledge and Insight** - All patients should be

   notified in person of the test result.

   If the result is negative:

- Provide the blood results with an explanation
- Reinforce safe sexual behaviours
- Discuss retesting in 3-6 months if there is recent high risk exposure
- Answer any questions
- Facilitate a referral to supportive services if the result is positive.
- Provide the blood test result and explain its meaning
- Clarify the difference between HIV and AIDS

If the setting is favourable these concepts should also be discussed

- Emphasize that patients can live with HIV/AIDS
- Encourage patients to share their diagnosis with a close family member or friend
- Discuss medical follow up options
- Discuss partner notification
- Reinforce safe sexual behaviours
- Emphasize availability for future contact
- Begin treatment literacy/readiness discussion

3. **Retesting -** The possibility that the person was in the period during an acute infection (before the sufficient production of HIV antibodies) should be ruled out with a repeat test in 3-6 months.
4. **Reduction of Spread** – The HIV positive individual is provided with additional psychosocial support and is linked to care. Several programmes are developed within the HIV/STI/ TB programme to ensure the patient is also retained in care. A treatment readiness assessment is done and patients are started on medications immediately after assessed as ready. Counselling on condom and contraceptive use as well as the disclosure of status is stressed to ensure there is a reduction of spread.
5. **Linkage to Care** – A Class 1 notification form should be completed for all patients with a positive HIV result. This is to be sent to the MO(H) within 24 hours. The Contact Investigator will then conduct a case investigation and the information should be sent simultaneously through the MO(H) to the regional and national levels. The client must be referred to a treatment site on the same day where a social worker will assess for barriers to linkage, ensuring appropriate services are offered. The client will then be assessed by members of the treatment team, including the physician and started on medications following the 'Test and Start' guidelines. **See the Protocols for Entry to Care, Linkage to Care and Retention and Recovery as well as the Clinical Management of HIV guidelines** for further details.

The HIV/AIDS Confidential Reporting Form

National Surveillance Unit, National Epidemiology Branch,
Arc Building, 15 Knutsford Boulevard, Kingston 5, St. Andrew
Email: surveillance@moh.gov.jm, Telephone 876-633-7937

HIV Confidential Reporting Form (CRF)

REASON FOR SUBMITTING CONFIDENTIAL REPORTING FORM		
◯ Initial HIV Diagnosis	◯ Pregnancy of HIV positive woman	◯ Risk History Update
◯ Advanced HIV Diagnosis	◯ ART Initiation	◯ Migration
◯ AIDS Diagnosis	◯ Treatment Regimen Change	
◯ Death Notification	◯ Viral suppression	

PATIENT'S DEMOGRAPHICS	
Patient's ID Type: ◯ Driver's License ◯ TRN ◯ Passport ◯ NIN	Patient's ID Number:
First Name:	Middle Name(s):
Last Name:	Pet Name(s):
Patient's Maiden Name (if applicable):	
Sex Assigned at Birth: ◯ Male ◯ Female	Gender: ◯ Male ◯ Female ◯ FTM ◯ MTF ◯ Other
Date of Birth: *DD/MM/YYYY*	Age:
Medical Record Number:	Is patient employed? ◯ Yes ◯ No
Occupation:	

Marital Status:			
◯ Married	◯ Visiting union	◯ Widowed	◯ Separated
◯ Common law	◯ Single	◯ Divorced	◯ Other (specify):

Is patient homeless? ◯ Yes ◯ No		
Country of Residence:	Parish (Jamaica):	Community:
House Number, Street Name:		
Landmark or directions to address:		
Phone Number:	Email Address:	

Highest level of Education completed:			
◯ No formal schooling	◯ Primary/All Age/Preparatory	◯ Tertiary /Post-secondary	◯ Other
◯ Basic	◯ Secondary/High School	◯ Vocational Skills Training	

Number of children under 15 years of age:	
Was client/patient/person ever deported? ◯ Yes ◯ No	
If yes, which country was the person deported from?	If yes, date deported (*MM/YYYY*):
MOTHER'S NAME:	
NEXT OF KIN:	
First Name:	Last Name:
Address: Lot/ Street/Community/Parish:	
Phone number of Next of Kin:	Email address of Next of Kin:

Relationship to the Patient:				
◯ Wife	◯ Father	◯ Sister	◯ Aunt	◯ Cousin
◯ Husband	◯ Son	◯ Brother	◯ Grandmother	◯ Friend
◯ Mother	◯ Daughter	◯ Uncle	◯ Grandfather	◯ Other

PATIENT'S RISK HISTORY						
	Ever			12 Months		
Date of Assessment: *DD/MM/YYYY*	**Yes**	**No**	**Unknown**	**Yes**	**No**	**Unknown**
Was patient incarcerated/ in prison?	○	○	○	○	○	○
Has patient used crack/cocaine?	○	○	○	○	○	○
Has patient used intravenous drugs?	○	○	○	○	○	○
Has patient had a sexually transmitted infection?	○	○	○	○	○	○
Has patient had genital ulcers/sores?	○	○	○	○	○	○
Has patient had sex with a commercial sex worker?	○	○	○	○	○	○
Has patient been a commercial sex worker?	○	○	○	○	○	○
Has patient had unprotected anal sex?	○	○	○	○	○	○
Has patient had multiple sex partners?	○	○	○	○	○	○
Was patient a victim of sexual assault?	○	○	○	○	○	○
Has patient had sex with a known person living with HIV (PLHIV)?	○	○	○	○	○	○
Has patient had transactional sex?	○	○	○	○	○	○
Did patient have perinatal exposure to HIV?	○	○	○	○	○	○
Has patient received a blood transfusion?	○	○	○	○	○	○
Blood Transfusion Date: *DD/MM/YYYY*						
Hospital where blood transfusion was received:						

STATUS OF PATIENT			
Date of Assessment: *DD/MM/YYYY*	**Yes**	**No**	**Unknown**
Does patient have generalised lymphadenopathy?	○	○	○
Does patient have generalised dermatitis?	○	○	○
Has patient had shingles?	○	○	○
Has patient experienced persistent cough for more than 4 weeks?	○	○	○
Has patient experienced fever for more than 1 month?	○	○	○
Does patient have chronic diarrhoea?	○	○	○
Has patient experienced recurrent pneumonia?	○	○	○
Does patient have Tuberculosis?	○	○	○
Pulmonary tuberculosis	○	○	○
Extra-pulmonary/disseminated tuberculosis	○	○	○
Does patient have central nervous system (CNS) disease/involvement?	○	○	○
Has patient had unintentional weight loss of more than 10% of body weight?	○	○	○
Has patient had Kaposi's Sarcoma?	○	○	○
Has patient been diagnosed with Pneumocystis carinii (jiroveci) pneumonia (PCP)?	○	○	○
Has patient had oral candidiasis?	○	○	○
Has patient had oesophageal candidiasis	○	○	○
Has patient had vaginal candidiasis?	○	○	○
Has patient had invasive cervical cancer?	○	○	○
Has patient had severe bacterial infection?	○	○	○
If Yes, please specify:			
Other clinical issues:			

Patient's Name:_____ Medical Record Number: _____

Post-test counselling:

Patient received post-test counselling: ◯ Yes ◯ No	Performed by: _____ (Enter Provider's Name)	Date of post-test counselling: DD/MM/YYYY

HIV SCREENING TEST

Date of Screening Test: DD/MM/YYYY

HIV Screening Test Type:

◯ Determine	◯ Colloidal Gold	◯ OraQuick	◯ Other (please specify)
◯ Uni-Gold	◯ ELISA	◯ Nucleic acid (NAT, PCR; viral load)	

HIV Screening Test Results:

◯ Positive	◯ Negative	◯ Indeterminate

How was test initiated?

◯ Provider-initiated	◯ Self-referral

Where was the test done?

◯ Antenatal Clinic	◯ Hospital	◯ Other Public Clinic	◯ STI Clinic
◯ Blood Bank	◯ Insurance	◯ Private Antenatal	◯ Private Pharmacy
◯ Home-based/self-test	◯ Mobile Clinic/Outreach	◯ Private Doctor	◯ Private Lab
Other (please specify): _____			

Date of Confirmatory Test: DD/MM/YYYY

Confirmatory HIV Test Type?

◯ Determine	◯ Colloidal Gold	◯ OraQuick	◯ Other (please specify)
◯ Uni-Gold	◯ ELISA	◯ Nucleic acid (NAT, PCR; viral load)	

Confirmatory HIV Test Result:

◯ Positive	◯ Negative	◯ Indeterminate

Laboratory where confirmatory HIV test performed:

◯ National Public Health Laboratory	◯ University Hospital of the West Indies	◯ Other (Specify):

BEHAVIOURAL PRACTICE OF PATIENT

With whom does patient have sex?

◯ Men only	◯ Women only
◯ Both men and women	◯ Refuses to answer

Key and vulnerable population characteristics of patient (Check all that apply):

◯ Men who have sex with men	◯ Prisoner/Prison history	◯ Intravenous Drug User
◯ Commercial sex worker	◯ Transgender	

Transmission Category:

◯ Sexual	◯ Blood Transfusion	◯ Haemophiliac
◯ IV Drug Use	◯ Vertical	

Patient's Sexual Contacts: Please complete contact listing on page 5.

Was this patient transferred from another treatment site?

◯ Yes ◯ No

Contact Tracing:

◯ Trace	◯ Do not contact trace	◯ Contact partners only

Patient's Name:_____ Medical Record Number: _____

Detailed Test Results		
CD4 Count (cells/µl):	Date of CD4 Count: *DD/MM/YYYY*	CD4/CD8 Ratio:
Viral Load (copies/ml):		Date viral load sample taken *DD/MM/YYYY*
Has patient ever taken antiretroviral medications (ARVs)? ◯ Yes ◯ No		ARV Start Date *DD/MM/YYYY*
Has patient ever taken pre-exposure prophylaxis (PrEP)? ◯ Yes ◯ No		If yes, what is the last date patient took (PrEP) *DD/MM/YYYY*
Has patient ever taken post-exposure prophylaxis (PEP)? ◯ Yes ◯ No		If yes, what is the last date patient took (PEP) *DD/MM/YYYY*
Other (Specify):		

ARV Line:

◯First line	◯Third line	◯Unknown
◯Second line	◯Salvage therapy	

Patient's Current Category:

◯ HIV	◯ Advanced HIV
◯ AIDS	◯ AIDS Death

Date of Onset of Symptoms: *DD/MM/YYYY*	Date diagnosed as Advanced HIV: *DD/MM/YYYY*
Date diagnosed as AIDS: *DD/MM/YYYY*	Date of death: *DD/MM/YYYY*

Cause of death:

i.	ii.	iii.
Immediate cause	Intermediate cause	Underlying cause

FOR PREGNANT WOMEN ONLY		
Is Patient Pregnant: ◯ Yes ◯ Unsure		Last Menstrual Period: *DD/MM/YYYY*
Gravida:		Parity:
Estimated Gestational Age: _____ weeks		Estimated Date of Delivery: *DD/MM/YYYY*
Clinic Medical Record Number:	Clinic Site:	Parish:

Status of Referral

◯ Not Referred	◯Referred	◯Refused Referral

Facility Referred To:	Date of First Antenatal Referral Appointment: *DD/MM/YYYY*

PHYSICIAN ATTENDING TO PATIENT
Name of Patient's Doctor:
Hospital/Clinic/Doctor's Office Address:
Phone Number:

Source(s) of information:

◯Patient	◯Next of Kin	◯Other
◯Medical record	◯Relative	

Comments:

Investigator's Details		
First Name:	Last Name	
Professional Group:		
Phone number:	Email Address:	
Name of Institution:		
Parish:	Community:	
Office Number/Street Name:		
Health Region: ◯ SERHA ◯ NERHA ◯ SRHA ◯ WRHA		
Date Investigation Completed: *DD/MM/YYYY*	Investigator's Signature:	
Name of Parish MO(H):	Signature of MO(H):	Date signed by MO(H): *DD/MM/YYYY*

National Surveillance Unit, National Epidemiology
Arc Building, 15 Knutsford Boulevard, Kingston 5, St. Andre
Email: surveillance@moh.gov.jm, Telephone 876-633-793

Patient's Sexual Contacts:

First Name (Include alias)	Last Name	Sex (M/F)	Relation	Address (include Parish)	Phone Number

Patient's Name: _____ Medical Record Number: _____

Definitions and Notes

- **Antiretroviral (ARV) Line**: Indicate ARV line patient is currently on/or last took - 1st, 2nd, 3rd, salvage therapy
- **Candidiasis**: Fungal infection with Candida spp that can be oral, oesophageal, and/or vaginal
- **Cause of death**: Recorded as i. Immediate, ii. Intermediate or iii. underlying causes.
- **Central Nervous System (CNS) Involvement**: Unexplained recent onset of seizures, dementia, toxoplasmosis, cytomegalovirus (CMV), cryptococcus, encephalopathy
- **Chronic Herpes Simplex**: Reported history of recurrent genital ulcer, blisters or crusts
- **Clinical Status**: Diagnosis should be confirmed by a physician
- **Commercial sex worker (CSW)**: Exchange of sex for cash as a main source of income
- **Cough (> 4 weeks)**: May be intermittent or persistent, productive or not
- **Date of Assessment**: Date on which assessment is being done for risk history or clinical status
- **Dermatitis**: Unexplained rash to multiple parts of the body
- **Diarrhoea (> 1 month)**: Three (3) or more watery stools in a 24-hour period
- **Education**: No formal schooling, Basic, Primary/All Age, Secondary/High School, Tertiary, Skills training, Other (specify)
- **Fever (> 1 month)**: On and off temperatures of 100.4 °F (38°C) or more than is temporarily alleviated by antipyretics or other medications
- **Generalised Lymphadenopathy**: Two or more sites with enlarged lymph nodes
- **Invasive Cervical Cancer**: History of or current cytological diagnosis of cervical cancer that has spread beyond the cervix – Stage 1a or greater
- **Kaposi's Sarcoma (KS)**: HIV-related Kaposi's sarcoma
- **Marital Status**: Married, common law, visiting union, single
- **Multiple Partners**: Persons who report having sex with more than one person within the past 12 months
- **PCP/PJP**: Current or past diagnosis of Pneumocystis carinii (jiroveci) pneumonia
- **Recurrent Pneumonia**: Repeated lower respiratory tract infections; Two or more episodes within a 1-year period
- **Severe Bacterial Infection**: Infection at any site of the body for which the patients had to be hospitalised for any period of time for treatment
- **Sexual Assault**: Anal or vaginal intercourse without consent; incident involved intimidation or threat or fear of violence
- **Shingles**: Recent or current diagnosis of the skin infection caused by the Varicella zoster virus
- **Transactional Sex**: Exchange of sex for food, goods or cash (but not as main source of income)
- **Transfer**: Indicate if the patient transferred in from another treatment site or private physician
- **Tuberculosis**: Confirmed TB case; If Yes: Pulmonary/Extra Pulmonary/Disseminated – select the appropriate condition(s)
- **Weight Loss (> 10%)**: Unintentional weight loss, i.e. patient has made no direct efforts to lose weight e.g., exercise or dieting

Patient's Name: _____ Medical Record Number: _____

GUIDELINES FOR THE INVESTIGATION OF HEPATITIS B

Hepatitis B is primarily transmitted through sexual contact. Therefore, contact investigators should be aware of all confirmed cases of hepatitis B infection to facilitate contact investigation. Individuals who test positive for the hepatitis B virus should receive counselling on essential information about the disease. The accompanying fact sheet and recommended counselling guidelines are provided to support contact investigators in this role.

Disease Information

Hepatitis B virus is passed from person to person by close contact with infected blood, saliva, semen, and vaginal fluids. The most common way for hepatitis B to spread is by sexual contact. Children born to mothers who have the virus in their blood are at particularly high risk of becoming infected.

After exposure to the virus, the average time until clinical illness develops is 60-80 days (normal range 45-180 days). Severity of the illness ranges from inapparent cases detectable only by liver function tests, to fatal cases involving acute hepatic necrosis. Case fatality is rare, approximately 1 % in hospitalized cases. The majority of cases will develop antibodies to the HBsAg and clear their infection in a few weeks or months. The infection is considered chronic when HBsAg persists for longer than 6 months after the resolution of symptoms.

Counselling HBsAg Positive Persons

The following steps should be taken when counselling persons who have been told that they are HBsAg positive:

1. **Confirm Results** - Be sure of the results of the test: confirm that the testing was for the hepatitis B surface antigen and that results were confirmed on a repeat sample of blood. If there is any doubt regarding the test results a repeat sample of blood should be collected and retested.
2. **Assess Knowledge and Insight** - The person should be counselled that the blood test has revealed that either he/she has been infected with the hepatitis B virus or that he/she was infected over 6 months ago and is a chronic carrier of the virus. Discussion of the risk factors for becoming infected and the common symptoms of the acute clinical illness may help the person define the time of infection and reinforce the reality of the situation. The long incubation as well as the possibility of mild to absent symptoms should be remembered and pointed out to persons who refuse to believe that they could be infected.
3. **Retesting** - The possibility that the person was in the period after acute infection before clearance of the HBsAg should be ruled out by a repeat test in 6 months. The potential for 10% of acute infections to go on to become chronic infections should be discussed, making the need for retesting in 6 months crucial.
4. **Reduction of Spread** - The individual should consider himself/ herself infectious to others for as long as HBsAg persists in his/her blood, and should be counselled as to the following issues:

    a) All his/her sexual contact(s) should be informed that they have been exposed to the hepatitis B virus and need to have a blood test to determine if they are infected. Susceptible ongoing sexual contacts (i.e., with no evidence of chronic, current, or past infection) should be advised to obtain a hepatitis B immunization series. Infected men/women and their sexual partners should be

educated to always use condoms during sexual contact.

b) All persons who live in close contact with an individual should be tested as well, regardless of age. Cuts or skin lesions should be covered to prevent the spread of infectious secretions or blood. Susceptible persons should be advised to obtain the hepatitis B immunization series.

c) The infected individual should not donate blood, plasma, body organs or tissues, or semen until it can be demonstrated that the HBsAg test is negative.

d) Individuals should be educated to always inform their dentists of their infectious status so that they can take protective measures to avoid infection.

e) An infected woman should be made aware that if she becomes pregnant during the period that she tests positive for the HBsAg, there is a high likelihood that she will pass the virus to her unborn child and that newborns are especially susceptible to developing a chronic infection (up to 70%) and the associated serious outcome of liver failure and liver cancer. If a pregnancy does occur in an actively infected woman, the infant should be given Hepatitis B immune globulin along with a hepatitis B vaccination series soon after birth.

The General Class 1 Notification Form

National Surveillance Unit, National Epidemiology Branch,
ARC Building, 15 Knutsford Boulevard, 2nd Floor, Kingston 5, St. Andrew
Email: surveillance@moh.gov.jm, Telephone 876-633-7937

The General Class 1 – Individual Notification Form

INITIAL DIAGNOSIS

Initial Diagnosis/Notification:

Date of Notification: *DD/MM/YYYY*	Date Patient Seen: *DD/MM/YYYY*	Case type: ◯ New Case ◯ Follow-up

Patient's Demographics

Patient's ID Type: ◯ Driver's License ◯ TRN ◯ Passport ◯ NIN	Patient's ID Number:
First Name:	Middle Name(s):
Last Name:	Pet Name(s):

Patient's Maiden Name (if applicable):

Sex Assigned at Birth: ◯ Male ◯ Female	Date of Birth: *DD/MM/YYYY*	Age:
Name of Facility Where Patient Seen:		Medical Record Number:
Country of Residence:	Community:	Parish (Jamaica):

House Number, Street Name:

Landmark or directions to address:

Phone Number:	Email Address:	Occupation:

Name of Workplace/School:

Address of Workplace/School:

Has the patient travelled overseas in the last 6 weeks? ◯ Yes ◯ No ***If yes, fill out the table below:***

Country Visited	City Visited	Date Arrived in Country	Date Departed Country
		DD/MM/YYYY	*DD/MM/YYYY*
		DD/MM/YYYY	*DD/MM/YYYY*
		DD/MM/YYYY	*DD/MM/YYYY*

Date returned to Jamaica ***DD/MM/YYYY***

Next of Kin

First Name:	Last Name:	Pet Name(s):
NOK ID Type: ◯ Driver's License ◯ TRN ◯ Passport ◯ NIN		NOK ID Number:

Address: Lot/ Street/Community/Parish:

Name of School/ Workplace:

Address of School/Workplace:

Phone number:	Email address:

Relationship to the Patient:

◯ Wife	◯ Son	◯ Uncle	◯ Cousin
◯ Husband	◯ Daughter	◯ Aunt	◯ Friend
◯ Mother	◯ Sister	◯ Grandmother	◯ Guardian
◯ Father	◯ Brother	◯ Grandfather	◯ Other (specify):

Clinical Profile of Patient

Has the patient experienced/Is the patient experiencing any of the following symptoms?

SYMPTOM	DATE OF ONSET	DURATION	SYPMPTOM	DATE OF ONSET	DURATION
◯ No Symptoms	*DD/MM/YYYY*		◯ Jaundice	*DD/MM/YYYY*	
◯ Abdominal Pain	*DD/MM/YYYY*		◯ Joint Pain	*DD/MM/YYYY*	
◯ Conjunctivitis	*DD/MM/YYYY*		◯ Myalgia	*DD/MM/YYYY*	
◯ Cough	*DD/MM/YYYY*		◯ Rash	*DD/MM/YYYY*	
◯ Diarrhoea	*DD/MM/YYYY*		◯ Seizures	*DD/MM/YYYY*	
◯ Eye Pain	*DD/MM/YYYY*		◯ Vomiting	*DD/MM/YYYY*	
◯ Fever	*DD/MM/YYYY*		◯ Weakness	*DD/MM/YYYY*	
◯ Haemoptysis	*DD/MM/YYYY*		◯ Other	*DD/MM/YYYY*	
◯ Headache	*DD/MM/YYYY*		Specify Other:		

Patient's Name: Medical Record Number:

Does the patient have any pre-existing conditions? ◯ Yes ◯ No ◯ Unknown

Please specify any pre-existing conditions

◯ Asthma	◯ Diabetes mellitus	◯ HIV/AIDS	◯ Malignancy
◯ Autoimmune	◯ Heart disease	◯ Hypertension	◯ Sickle cell disease
◯ Chronic obstructive pulmonary disease (COPD)	◯ Kidney disease	◯ Liver dysfunction	◯ Other Specify Other:

Vaccination Status:

Type of Vaccine	Number of doses	Date of last dose	Source of Vaccination Information (Vaccination card, Health service record, Verbal)
		DD/MM/YYYY	
		DD/MM/YYYY	
		DD/MM/YYYY	
		DD/MM/YYYY	
		DD/MM/YYYY	

Was patient admitted to hospital? ◯ Yes ◯ No	If yes, date of admission DD/MM/YYYY
Name of hospital:	Ward:
Was patient discharged from hospital? ◯ Yes ◯ No	If yes, date discharged DD/MM/YYYY
Is patient deceased? ◯ Yes ◯ No	If yes, date of death DD/MM/YYYY

Sample and Lab Information

Was a sample taken? ◯ Yes ◯ No

If yes, what specimen was taken?

◯ CSF	◯ Nasopharyngeal Swab	◯ Sputum	◯ Tissue
◯ Blood Smear	◯ Naso-/Oro-/ Pharyngeal Swab	◯ Stool	◯ Other (specify):
◯ EDTA Blood	◯ Serum	◯ Urine	

What date was the specimen taken? DD/MM/YYYY

Which lab is the sample being sent to?

◯ National Public Health Laboratory ◯ University of the West Indies ◯ Other

Specify Other Laboratory:

Lab Test(s) Requested:

◯ Antigen	◯ PCR	◯ Culture	◯ Microscopy
◯ ELISA	◯ Western Blot	◯ Other (specify):	

Lab result(s): ◯ Positive ◯ Negative ◯ Indeterminate

Notifier's Comments:

Notifier's Details

Date of Reporting (DD/MM/YYYY): /

First Name:	Last Name
Professional Group:	
Phone number:	Email Address:
Name of Institution:	
Office Number/Street Name:	
Community:	Parish:

Health Region: ◯ SERHA ◯ NERHA ◯ SRHA ◯ WRHA

Name of Parish MO(H):	Signature of MO(H):	Date signed by MO(H): DD/MM/YYYY

GUIDELINES FOR THE INVESTIGATION OF HTLV

HTLV (Human T-cell Lymphotropic Virus) is one of the infections routinely screened for by the Blood Bank. It was the first human retrovirus to be isolated and is known to cause two serious conditions: adult T-cell leukemia/lymphoma (ATL) and a chronic neurological disorder called HTLV-associated myelopathy/tropical spastic paraparesis (HAM/TSP).

Despite this, most individuals infected with HTLV remain asymptomatic and appear to be healthy carriers.

Like HIV and HBV, HTLV can be transmitted through sexual contact. However, limited information is available about the specific risk factors for heterosexual transmission. Individuals who test positive for HTLV should receive counseling to ensure they understand key facts about the virus, its implications, and potential health risks. The following fact sheet and counseling guidelines are provided to help the contact investigator in their role.

Disease Information

HTLV is transmitted through close contact with infected bodily fluids, including blood, saliva, semen, and vaginal secretions. Sexual contact is the most common mode of transmission.

Children born to mothers with the virus in their blood are at particularly high risk of infection.

Following exposure, adult T-cell leukemia/lymphoma (ATL) is believed to develop after a long latent period—often around 40 years. The risk of developing ATL appears to be highest in individuals who were infected during infancy.

Counselling HTLV1 Positive Persons

The following steps should be taken when counselling persons who are HTLV positive:

1. **Confirm Results** - Be sure of the results of the test: confirm that the testing was for the HTLV virus. If there is any doubt regarding test result, a repeat sample of blood should be collected and retested.
2. **Assess Knowledge And Insight** - The person should be counselled that the blood test has revealed that he/ she has been infected with the HTLV virus, or that he/she may have been infected many years ago, and is a chronic carrier of the virus.
3. **Reduction of Spread** - The individual should consider himself/herself infectious to others and should be counselled as to the following issues:
    a. All his/her sexual contact (s) should be informed that they have been exposed to the HTLV virus and need to have a blood test to determine if they are infected. Infected persons should be educated to always use condoms during sexual contact.
    b. Female sex contacts, in particular, should be so informed since male-to-female transmission in this condition is more efficient than female-to-male transmission.
    c. The infected individual should not donate blood, plasma, body organs, tissues, or semen.
    d. An infected woman should be made aware that if she becomes pregnant there is a high likelihood

that she may pass the virus to her unborn child if she breast feeds, and that infected newborns are especially susceptible to developing ATL in later life.

4. **Follow-up**

5. All patients should be referred to the physician for an assessment. If symptomatic, the patient is to be referred to the Medical Out-patient Department (MOPD).

Social Services

In the course of investigating STI-related cases such as those outlined previously, it may become necessary to contact social services. The following services are available:

SOCIAL SERVICES - HOTLINE NUMBERS

CHILD PROTECTION AND FAMILY SERVICES AGENCY
(CPFSA)
876-878-2882
211

Child Protection and Family Services Agency
Protecting Children, Empowering Families, Securing the Future
REPORT CHILD ABUSE 211 REPORT CHILD ABUSE 211

NO EXCUSE FOR ABUSE
Female callers:
876-553-0372

Male callers
876-553-0387

1-888-PROTECT
1-888-777-8328



SAFE SPOT
(For Children)

WhatsApp:
876-439-5199

Safe Spot
A CHILD & TEEN HELPLINE

VICTIM SERVICES DIVISION
876-946-0663
OR
876-946-9287
Toll Free:
888-587-8423

MINISTRY OF JUSTICE
BALANCING RIGHTS & RESPONSIBILITIES

WOMAN INC. CRISS CENTRE

KINGSTON
(24 HOURS)
876-929-2997

ST. JAMES
Monday - Friday
9:00am - 5:00pm
876-952-9533

WOMAN INC.

CHOOSE LIFE
(Suicide Prevention)
876-920-7924

876-856-2966
(WhatsApp)

CLI
CHOOSE LIFE INTERNATIONAL
HELPING PEOPLE LIVE

MINISTRY OF HEALTH & WELLNESS
MENTAL HEALTH & SUICIDE PREVENTION
888-NEW -LIFE
888-639-5433

MINISTRY OF HEALTH & WELLNESS

RISE LIFE MANAGEMENT SERVICES
(Substance Use)

876-967-3777

Reaching Individuals through Skills & Education
Rise
Life Management Services

CODE AND NUMBER ASSIGNED TO REGIONS AND PARISHES

Contact investigators are assigned unique identifiers per region. These numbers facilitate the identification of the CI on reports and the regions from which the reports originate.

HEALTH REGION	CODE	NO.
South east regional health authority	SERHA	1
NORTHEAST REGIONAL HEALTH AUTHORITY	NERHA	2
SOUTHERN REGIONAL HEALTH AUTHORITY	SRHA	3
WESTERN REGIONAL HEALTH AUTHORITY	WRHA	4
PARISH	**CODE**	**NO.**
Kingston and St Andrew*	KSA	1
St Thomas	STT	3
Portland	POR	4
St Mary	STM	5
St Ann	STA	6
Trelawny	TRE	7
St James	STJ	8
Hanover	HAN	9
Westmoreland	WES	10
St Elizabeth	STE	11
Manchester	MAN	12
Clarendon	CLA	13
St Catherine	STC	14

*St Andrew - 2 but normally grouped with Kingston as 1